

MINISTRY OF LABOUR AND
SOCIAL PROTECTION

STATE DEPARTMENT FOR SOCIAL PROTECTION
AND SENIOR CITIZEN AFFAIRS

COMMUNITY BASED
INCLUSIVE DEVELOPMENT

STRATEGIC FRAMEWORK

2026 - 2030

Table of Content

Acronyms	1
Defination of terms	2
FOREWORD	4
EXECUTIVE SUMMARY	5
Executive Summary	8
CHAPTER ONE	9
1.1 Background	10
1.2 Vision	11
1.3 Mission	12
1.4 Core Values	12
1.5 Objective of the Strategic Framework	12
1.6 Key Result Areas (KRAs)	12
1.7 Strategic Objectives	13
1.8 Strategic Challenges, Objectives, and Strategies	13
CHAPTER TWO	18
2.1 Situation Analysis	19
2.2 SWOT Analysis	19
2.3 PESTEL Analysis	19
2.4 Management structures in CBID programmes	21
CHAPTER THREE	23
3.1 ACCESSIBLE HEALTH SERVICES	24
3.1.1 Role of CBID in Health	24
Desired Outcome	24
3.1.2 Elements in the Health Component	25
A) Health promotion	25
B) Prevention	26
C) Medical care	26
D) Rehabilitation	27
E) Assistive devices and technologies	27
3.2 INCLUSIVE EDUCATION AND TRAINING	28
Role of CBID in Education	28
Desired Outcome	28
3.2.1 Elements in the Education Component	29
A) Early childhood care and education	29
B) Primary Education	29
C)Secondary and higher education	30

D)Non-formal Education	30
E) Lifelong learning	31
3.3 IMPROVED LIVELIHOOD, SOCIAL PROTECTION, AND COMMUNITY DEVELOPMENT ..	31
3.3.1 Role of CBID in Livelihood	32
Desired Outcome	32
3.3.2 Elements in the Livelihood Component	33
A) Skills development	33
B) Self- development	33
C)Wage employment	34
D)Financial services	34
E) Social protection	34
3.4 SOCIAL INCLUSION	35
3.4.1 Role of CBID in Social Component	35
Desired Outcome	35
3.4.2 Elements in the Social Component	36
A) Personal assistance	36
B) Relationships, marriage, and family	36
C)Culture and arts	37
D)Recreation, leisure, and sports	37
E) Justice	38
3.5 EMPOWERMENT OF PERSONS WITH DISABILITIES AND THEIR FAMILIES	38
3.5.1Role of CBID in Empowerment	39
Desirable Outcomes	39
3.5.2 Elements in the Empowerment Component	39
A) Advocacy and Communication	39
B) Community Mobilization	39
C)Political Participation	40
D)Community groups	40
E) Organizations of and for Persons with Disabilities	41
ANNEX I: IMPLEMENTATION MATRIX FOR 2026-2030	42
ANNEX II: MONITORING AND EVALUATION/ RESULTS MATRIX FOR 2026 -2030	70

LIST OF ACRONYMS

CBR	Community Based Rehabilitation
OPDS	Organization of and for Persons with Disabilities
CBID	Community Based Inclusive Development
WHO	World Health Organization
CRPD	Convention on the Rights of Persons with Disabilities
TAF	The Action Foundation
CBM	Christian Blind Mission
NCPWD	National Council for Persons with Disabilities
SDGS	Sustainable Development Goals
UNCRPD	UN Convention on the Rights of Persons with Disabilities
UN	United Nations
AGPO	Access to Procurement Opportunities
UHC	Universal Health Care
VRCS	Vocational Rehabilitation Centres'
COK	Constitution of Kenya
AT	Assistive Technology

DEFINITION OF TERMS

Affirmative Action: Active effort to favor those who suffer from discrimination especially in relation to education or employment.

Assistive device and Technology: Includes Implements, tools, equipment, taped texts, audio, visual and pictorial recording, braille equipment's and material, tactile equipment, orthopedic appliances, software's and other devices and machines for persons with disabilities for their socio-cultural, economic, civil, political wellbeing of persons with disabilities.

Capacity-building: developing and strengthening the skills, knowledge, tools, and resources that individuals, organizations, or communities need to perform effectively and sustainably.

Caregiver: a person who tends to the needs or concerns with short- or long-term limitations due to illness, injury or disability.

Community Group: means a voluntary association of individuals from the same community which is self-organized for a common purpose aimed at improving the livelihood of the group members or for a community benefit and includes a special interest group, community project, and community-based organization but shall not include a Public Benefit Organization, Non-Governmental Organization or groups formed to champion a political cause or contrary to public policy;

Community-Based Inclusive Development (CBID) Advisory Committee, Kenya: The Community-Based Inclusive Development (CBID) Advisory Committee in Kenya is a multi-stakeholder body established to guide, coordinate, and strengthen efforts towards promoting the rights, participation, and inclusion of persons with disabilities in community life.

Community-Based Inclusive Development is a strategy to promote inclusion, rights and equal opportunities for persons with disabilities to ensure inclusive development.

Disability: Includes any physical, sensory, mental, psychological or other impairment, condition or illness that has, or is perceived by significant sectors of the community to have, a substantial or long-term effect on individuals' ability to carry out day to day activities (e.g. feeding, toileting, movement, grooming)

Discrimination: any distinction, exclusion or restriction on the basis of disability which has the purpose or effect of impairing or nullifying the recognition, enjoyment or exercise on an equal basis with others, of all human rights, and fundamental freedoms in the political, economic, social, cultural, civil or any other field. it includes all forms of discrimination including denial of reasonable accommodation.

Habilitation: refers to support services that help a person with disabilities keep, learn, or improve skills and functioning for daily living. These habilitative services may include physical and occupational therapy, physiotherapy, speech-language therapy, assistive devices and technology, and other services for persons with disabilities.

Inclusion: process of adjusting home, school, and society so that all the individuals, regardless of their differences, can have the opportunity to interact, play, learn, work and experience the feeling of belonging, and experiment to develop in accordance with their potentials and difficulties

Inclusive education: means an approach where learners and trainees with disabilities are provided with appropriate educational interventions within regular institutions of learning with reasonable accommodation and support

Meaningful participation: it is enabling Persons with disabilities to actively and effectively engaging in decision making process across all levels from design to evaluation, ensuring that they are not excluded due to barriers.

Organizations for Persons with Disabilities means associations, groups, non-governmental organizations, or societies formed for the purposes of rendering services to persons with disabilities.

Organizations of Persons with Disabilities means associations, societies or other membership groups led and controlled by persons with disabilities to represent the rights and interests of persons with disabilities.

Personal Assistance: A personal assistant provides practical services to persons with disabilities in personal and legal tasks so they can live independently in the community. A personal assistant can perform a variety of functions ranging from daily activities such as cooking and assistance in mobility to banking and paying bills. There may be more than one personal assistant at any given time performing the same or different tasks.

Persons with disabilities means those who have long term physical mental intellectual or sensory impairments which in interaction with various barriers may hinder their full and effective participation in society on an equal basis with others

Rehabilitation: is set of interventions designed to reduce disability, optimize functioning and improve health conditions of a person to enable easy interaction with their environment. Its rebuilding or re-learning skills that were lost.

Respite Services are services provided in a facility or by volunteers to offer short term relief for primary caregivers. It can be arranged for just an afternoon or for several days or weeks.

Social protection: Noncontributory measures aimed at preventing and mitigating economic and social vulnerabilities to poverty, risks, contingencies, and shocks experienced throughout a person's life cycle.

Technical and vocational education and training aspects of the educational process: involving in addition to general education, the study of technologies and related sciences and the acquisition of practical skill, attitudes, understanding, and knowledge relating to occupants in various sectors of economic and social life.

FOREWORD



It gives me great pride to present the Community-Based Inclusive Development (CBID) Framework, 2026–2030, a milestone document that underscores Kenya's steadfast commitment to building an inclusive society where persons with disabilities are empowered to thrive. This Framework comes at a critical moment when our nation is prioritizing equity, social protection, and sustainable development in line with the Constitution of Kenya 2010, Vision 2030, and the Sustainable Development Goals (SDGs).

The CBID Framework recognizes that true development cannot be achieved if any segment of society is left behind. It therefore provides a practical blueprint for ensuring that persons with disabilities, their families, and communities are meaningfully engaged in shaping their futures. It seeks to strengthen empowerment, promote social inclusion, enhance access to education, improve health care, and expand livelihood opportunities for persons with disabilities across all counties and communities in Kenya.

The Government of Kenya, working with County Governments, the private sector, Development Partners, Organizations of Persons with Disabilities, and communities, will ensure that the strategies outlined in this Framework are implemented with dedication and accountability. This multisectoral collaboration will guarantee that inclusion is not just a policy aspiration but a lived reality for all persons with disabilities and their families.

As a Ministry, we reaffirm our unwavering commitment to championing the rights, dignity, and potential of persons with disabilities. We shall continue to strengthen policies, mobilize resources, and monitor progress to ensure that this Framework delivers measurable outcomes that transform the lives of persons with disabilities across Kenya.

I call upon all stakeholders to embrace this Framework with a spirit of partnership and shared responsibility. Together, let us build a society where no one is left behind, where every citizen has equal opportunity, and where disability is not a limitation but a source of strength and diversity for our nation.

**Dr. Alfred Mutua, CBS,
Cabinet Secretary
Ministry of Labour and Social Protection**

ACKNOWLEDGMENT



The successful development of the Community-Based Inclusive Development (CBID) Framework 2026–2030 is the result of a highly collaborative and inclusive process. On behalf of the State Department for Social Protection and Senior Citizen Affairs, I wish to express my deepest appreciation to all institutions and individuals who contributed to the realization of this document.

Special recognition goes to, the United Disabled Persons of Kenya (UDPK) and all Organizations of Persons with Disabilities including Differently Talented Society of Kenya, Kenya Association for the Intellectually Handicapped, Endorois Indigenous Women Empowerment Network, Sight for Relief, Black Albinism, Deaf Empowerment Kenya and civil society actors who provided invaluable insights drawn from lived experiences. Your voices shaped the priorities, strategies, and interventions contained herein, ensuring that this Framework truly reflects the aspirations of the disability community.

I am equally grateful to the government agencies and departments that played a pivotal role in this process. In particular, I wish to recognize the dedicated staff from the Ministry of Health, Ministry of Education, the Kenya Institute for Special Education, and the National Fund for the Disabled of Kenya. Their unwavering commitment and support ensured that the key components of Education, Health, and Empowerment were meaningfully and comprehensively included into the document advancing dignity, equality, and opportunity for all persons with disabilities.

We acknowledge The Action Foundation for their unwavering support in terms of technical expertise, financial resources, and innovation. Your solidarity demonstrates the power of partnerships in advancing the inclusion agenda.

Lastly, I wish to commend the dedicated technical team within the Ministry of Labour and Social Protection namely the Directorate of Social Development and the National Council for Persons with Disabilities (NCPWD) whose tireless effort, commitment, and professionalism made the drafting and finalization of this Framework possible.

It is our hope that the CBID Framework will serve as a guiding light for inclusive development in Kenya, advancing dignity, equality, and opportunity for all persons with disabilities.



**Joseph Motari, CBS,
Principal Secretary
State Department for Social Protection and Senior Citizen Affairs**

Community-Based Inclusive Development (CBID) Advisory Committee, Kenya

The Community-Based Inclusive Development (CBID) Advisory Committee in Kenya is a multi-stakeholder body established to guide, coordinate, and strengthen efforts towards promoting the rights, participation, and inclusion of persons with disabilities in community life. Rooted in the principles of the UN Convention on the Rights of Persons with Disabilities (CRPD), the Sustainable Development Goals (SDGs), the African Disability Protocol, and the Persons with Disabilities Act (2025), the committee ensures that CBID initiatives are people-centered, rights-based, and aligned with national policies and frameworks.

The main purpose of the CBID Advisory Committee is to provide strategic direction, oversight, and technical guidance in the implementation and scaling up of CBID in Kenya. It serves as a platform for collaboration between government, Organizations of Persons with Disabilities (OPDs), Development Partners, and communities, thereby ensuring that CBID contributes to inclusive social and economic development.

The roles of the committee include providing policy and technical advice on CBID implementation, promoting alignment of CBID programs with CRPD, SDGs, and national disability policies, and strengthening coordination among government agencies, OPDs, civil society, and development partners. The committee also supports resource mobilization and partnerships for sustainable CBID initiatives, facilitates capacity building of stakeholders to enhance disability-inclusive approaches, and monitors, evaluates, and documents best practices and lessons learned.

The committee draws its membership from a broad representation of stakeholders. These include the State Department for Social Protection, Senior Citizens Affairs, the National Council for Persons with Disabilities (NCPWD), Organizations of Persons with Disabilities (OPDs), county governments, civil society organizations, development partners, academia, research institutions, community-based organizations, and the private sector.

Through its work, the CBID Advisory Committee is expected to enhance policy coherence and effective implementation of CBID in Kenya, strengthen partnerships and collaboration across stakeholders, and increase the participation and empowerment of persons with disabilities at the community level. It will also facilitate the scaling up of best practices in inclusive community development, ultimately contributing to sustainable and coordinated disability-inclusive development that supports Kenya's broader social and economic transformation.

EXECUTIVE SUMMARY

The Community-Based Inclusive Development (CBID) Framework 2026 – 2030 is a comprehensive guide for advancing disability inclusion in Kenya. It builds on the Constitution of Kenya 2010, the Persons with Disabilities Act 2025, Vision 2030, and international commitments such as the UN Convention on the Rights of Persons with Disabilities (UNCRPD) and the Sustainable Development Goals (SDGs). The Framework provides a practical roadmap for empowering persons with disabilities, enhancing their participation in society, and ensuring they are not left behind in national development.

Chapter One introduces the context and rationale for the Framework. It highlights the challenges faced by persons with disabilities, including stigma, discrimination, limited access to services, and socio-economic marginalization. The chapter underscores the government's commitment to equality, dignity, and participation while explaining the philosophy of CBID as a community-driven, rights-based approach that brings together persons with disabilities, families, and communities in driving inclusive development.

Chapter Two sets out the guiding principles, vision, mission, and objectives of the Framework. It emphasizes participation, equity, accountability, partnership, and sustainability as core values. The chapter outlines how CBID aligns with Kenya's policy frameworks, including the Persons with Disabilities National Policy 2024, and links with broader national and global agendas.

Chapter Three presents the strategic approach of the Framework, organized into five Key Result Areas. These include improved access to health services, access to inclusive education and training, improved livelihoods and social protection, enhanced social inclusion and empowerment of persons with disabilities and their families. Each area is supported by strategic objectives, activities, outputs, indicators, and lead implementing agencies, providing a clear roadmap for action.

Chapter Four elaborates the institutional arrangements for implementation. It defines the roles of the Ministry of Labour and Social Protection, the State Department for Social Protection and Senior Citizen Affairs, the National Council for Persons with Disabilities, County Governments, and other actors. It stresses the importance of multi-sectoral collaboration involving civil society, Organizations of Persons with Disabilities, the private sector, development partners, and community structures.

Chapter Five focuses on financing and resource mobilization. It highlights the need for government budgetary allocation, development partner support, and innovative financing mechanisms, while calling for mainstreaming of disability issues across all sectors to ensure sustainability.

Chapter Six outlines the monitoring, evaluation, and reporting framework. It provides mechanisms for tracking progress through indicators, data collection systems, and periodic reviews. This ensures accountability and evidence-based policy adjustments to achieve meaningful outcomes for persons with disabilities.

The Framework concludes by reaffirming Kenya's commitment to disability inclusion as both a development and human rights priority. It emphasizes that the realization of the CBID vision requires collective responsibility and partnership across government, communities, and stakeholders. By implementing this Framework, Kenya moves closer to creating a society where all persons, regardless of disability, enjoy dignity, equality, and the opportunity to fulfill their potential.

CHAPTER 01

1.1 Background

The current world population is estimated to be eight billion persons, out of which sixteen percent (16%) is estimated to be persons with disabilities (WHO and WB, 2011). Persons with disabilities constitute 20% of the world poorest people, with the number of women with disabilities being higher than that of men, further, 80% of persons with disabilities live in developing countries.

According to the Kenya Population and Housing Census of 2019 persons with disabilities account for 2.2% of Kenyans aged 5 and above (0.918 million) of the general population. The Kenya National Bureau of Statistics indicates that 90% of Kenya's population have lived experience directly or indirectly as persons with disabilities.

Kenya's earliest recorded initiative for organized care and provision of services to persons with disabilities goes back to the missionary era. In 1946, the Salvation Army Church established a programme to rehabilitate men blinded during the Second World War. The programme later became the country's first school for the blind marking the commencement of provision of formal education for blind children in Kenya and East Africa.

The mainstream churches: Catholic, Presbyterian, Anglican and Methodist followed this example by establishing schools and institutions for children with visual, hearing, mental and physical disabilities in various parts of the Country where they had their missions. The Government supported these efforts by providing an enabling environment and practical support.

Community Based Rehabilitation (CBR) was initiated in the early 1980s, due to the failure of the conventional system of rehabilitation then prevalent in many developing countries, and the need for an alternative approach (Helander, 1993). The focus was on coverage of rehabilitation services for persons with disabilities living in rural areas in developing countries, at an affordable cost. The main strategy was the transfer of skills to local people, including families (WHO, 1989).

From the mid-nineties onwards, there were major changes in CBR, captured in the Joint Position Paper of WHO, ILO and UNESCO (2004) that defined CBR as "a strategy within general community development for the rehabilitation, equalization of opportunities and social inclusion of all persons with disabilities. It is implemented through the combined efforts of persons with disabilities themselves, their families, organizations and communities, and the relevant governmental and non-governmental agencies. Today, CBR is understood as a strategy to promote inclusion, rights and equal opportunities for persons with disabilities to ensure inclusive development.

Kenya signed and ratified the Convention on the Rights of Persons with Disabilities in 2007 and 2008 respectively. The Convention seeks to promote, protect and ensure the full and equal enjoyment of all human rights and fundamental freedoms by all persons with disabilities and to promote respect for their inherent dignity.

The Community Based Rehabilitation Guidelines of WHO (2010), based on the principles of the UN-CRPD, and developed with the active participation of Organizations of and for Persons with Disabilities (OPDs), United Nations agencies, governments, and civil society organisations, are in recognition of the importance of CBR as a world-wide movement. The Guidelines attempt to synthesise experiences from across the world to provide a unified understanding of the concept and principles of CBR. The Guidelines provide a structure that can be contextualized by CBR planners and practitioners in Kenya on key activities that can be implemented according to their local context, needs and resources. It further has been domesticated within this strategy by the Government of Kenya on the key programmes that can lead to Community Based Inclusive Development.

The strategic plan has endeavored to mainstream the relevant Sustainable Development Goals (SDGs), targets and indicators within its thematic areas and interventions. These SDGs include:

- SDG 4.5 By 2030, eliminate gender disparities in education and ensure equal access to all levels of education and vocational training for the vulnerable, including persons with disabilities.
- SDG 8.5 By 2030, achieve full and productive employment and decent work for all women and men, including for young people and persons with disabilities, and equal pay for work of equal value.
- SDG 10.2 By 2030, empower and promote the social, economic, and political inclusion of all, irrespective of age, sex, disability, race, ethnicity, origin, religion or other status.
- SDG 11.2 By 2030, provide access to safe, affordable, accessible, and sustainable transport systems for all, improving road safety, notably by expanding public transport, with special attention to the needs of those in vulnerable situations, women, children, persons with disabilities and older persons.
- SDG 11.7 By 2030, provide universal access to safe, inclusive, and accessible, green, and public spaces, for women and children, older persons and persons with disabilities.
- SDG 17.18 By 2020, enhance capacity-building support to developing countries, including least developed countries and small island developing States, to significantly increase the availability of high-quality, timely and reliable data disaggregated by income, gender, age, race, ethnicity, migratory status, disability, geographic location, and other characteristics relevant in national contexts.

In light of the foregoing, the Government of Kenya recognizes the need for a strategy on community-based inclusive development as a commitment towards addressing the rights, dignity, and protection of this vulnerable category of the society in line with the Constitution of Kenya that has several Articles focusing on specific components of the CBR.

On 10th June 2008, Kenya also launched the Kenya development program dubbed *The Kenya Vision 2030* aiming to raise the average standard of living in Kenya to middle income by 2030. The Kenya vision 2030 has three main pillars namely Economic, Social and Political. Kenya aims to improve the prosperity of all Kenyans through an economic development programme, covering all the regions of Kenya; to build a just and cohesive society with social equity in a clean and secure environment and to realize a democratic political system founded on issue-based politics that respects the rule of law, and protects the rights and freedoms of every individual in Kenyan society. This strategy makes special provisions for persons with various disabilities and previously marginalized communities.

These pillars speak to all the five components of community-based rehabilitation namely; Empowerment, Livelihood, Social, Education and Health. This document recognizes the challenges and barriers concerning implementing community-based rehabilitation within the five components and the strategies to address these challenges hence ensuring that the major objectives of CBR are achieved.

To further enhance inclusivity of persons with disabilities within communities, the government of Kenya seeks to synergize Community based rehabilitation with community based inclusive development. In view of this, the strategy adapts the name Community based inclusive development.

1.2 Vision

Persons with disabilities and their families live with dignity, independence, and equal opportunities as active members in the community.

1.3 Mission

To promote inclusive society where persons with disabilities and their families meaningfully participate in all aspects of life, enjoy equal rights, and access opportunities on an equal basis with others.

1.4 Core Values

- Inclusion
- Dignity
- Non-Discrimination
- Supportiveness
- Transparency and Accountability
- Honesty
- Respect

1.5 Objective of the Strategic Framework

Promote a common understanding and guide all stakeholders in implementing CBID programmes in line with the standards and principles of CRPD ensuring persons with disabilities enjoy their rights, access opportunities and actively participate in the community on an equal basis with others.

1.6 Key Result Areas (KRAs)

Five Key Results Areas (KRAs) have been identified as per the Community-Based Rehabilitation Components, which if fully implemented, will develop, and strengthen Community Based Inclusive Development programmes in Kenya. The health component covers promotion, prevention, medical care, rehabilitation and assistive devices. Education component covers Early childhood, primary secondary and higher, non-formal and lifelong learning. Livelihood component covers skills development, self-employment, wage employment, financial services and social protection. Social component covers personal assistance, relationships, marriage and family, culture and arts, recreation, leisure, sports and justice. Empowerment covers advocacy and communication, community mobilization, political participation, Community groups, Organizations of Persons with Disabilities formerly (Disabled Persons Organizations).

The Key Result Areas of this Strategic Plan include:

- Accessible health services.
- Inclusive Education and training.
- Improved livelihood, social protection, and community development.
- Social Inclusion and
- Empowerment of persons with disabilities and their families.

Strategic Objectives

1. To promote access to inclusive and comprehensive health services to persons with disabilities including rehabilitation, assistive technologies and sexual and reproductive health services, while strengthening preventive and promotive care to uphold the right to the highest attainable standard of health.
2. Ensure equitable access to inclusive education from early childhood through primary, secondary, higher levels and non-formal education and lifelong learning enabling persons with disabilities to realize their full potential, cultivate dignity and self-worth, and actively participate in society.
3. To enhance the economic independence, security, and social inclusion of persons with disabilities by ensuring access to skills development, self-employment opportunities, decent wage employment, inclusive financial services, and comprehensive social protection measures.
4. To ensure the full inclusion of persons with disabilities in family and community life through access to personal assistance, opportunities for relationships, marriage and family, participation in culture, arts, recreation and sports, and equal access to justice, enabling them to enjoy their rights and contribute actively to society.
5. To empower persons with disabilities through advocacy and communication that raises awareness and shifts attitudes; community mobilization that strengthens solidarity and action at the grassroots; political participation that enhances representation and influence in governance; and the development of Community groups and organizations of persons with disabilities as sustainable platforms for leadership, self-representation, and inclusive development.

1.7 Strategic Challenges, Objectives, and Strategies

The Strategies, Strategic Objectives and Challenges under the five Key Results Areas are presented below.

Key Result Area I: Accessible health services.	
Strategic challenges	a) Lack of financial assistance, health interventions such as assessments, treatments, and medications b) Inaccessible mode of transport, inaccessible buildings, and medical equipment. c) Lack of proper communication tools, information and sensitization on matters relating to health information. d) Lack of medical technique by health workers in handling persons with disabilities. e) Limited knowledge on disability inclusion by health care workers, community health promoters, community health assistants f) Limited knowledge on disability medical assessment and categorization. g) Poor attitudes and knowledge of health matters on persons with disabilities by community members. h) Poor knowledge, and attitudes of persons with disabilities about general health care and services. i) Poor health seeking behaviors.
Strategic Objective: To promote access to inclusive and comprehensive health services to persons with disabilities including rehabilitation, assistive technologies and sexual and reproductive health services, while strengthening promotive and preventive care to uphold the right to the highest attainable standard of health.	

Strategies	a) Adopting community approaches to address health problems that have strong environmental, socioeconomic, and political components. b) Improve accessibility for persons with disabilities to access health services. c) Health promotion materials and programs designed or adapted to meet the specific needs of persons with disabilities and their families. d) Build capacity of health workers on disability medical handling techniques e) Sensitize the community members to bring about changes in perceptions, in attitudes towards persons with disabilities. f) Provide training for persons with disabilities, in partnership with the health sector, to increase their knowledge of healthcare services g) Pre- and in-service Training of health care workers on disability medical assessment and categorization h) Strengthen the capacity of health care workers on disability inclusion. i) Promote access to promotive and preventative health care services including early identification to persons with disabilities. j) Developing inclusive communication strategy for healthcare workers.
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Key Result Area II: Inclusive Education and Training

Strategic challenges	a) Inaccessible learning institutions, amenities, and limited availability of specialized instructional materials and technologies for learners with disabilities. b) Parents, guardians, and caregivers from low-income and poverty-stricken areas are unable to pay school fees and purchase assistive devices. c) Long distances between home and school and inaccessible public transport facilities. d) No adapted curriculum for all learners with disabilities e) Teachers and other education facilitators not equipped to handle learners with various disabilities. f) Traditional assessment methods do not accurately measure the progress of learners with disabilities. g) Negative attitudes and stereotypes about disabilities h) Lack of internal inclusive policies and adequate resources, training, and support for teachers in schools and institutions.
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Strategic Objective: Ensure equitable access to inclusive education from early childhood through primary, secondary, higher levels and non-formal education and lifelong learning enabling persons with disabilities to realize their full potential, cultivate dignity and self-worth, and actively participate in society.

Strategies	a) Accessible infrastructure and transportation systems. b) Provide financial assistance to families of learners with disabilities which may include subsidizing assistive devices and other school-related expenses. c) Develop an adapted curriculum and offer specialized Support Services, including speech therapy, occupational therapy, and counselling. d) Provide comprehensive training for educators on inclusive teaching practices and strategies for supporting learners with disabilities. e) Invest in early intervention programs. f) Equitable Assessment and Evaluation of the Adapted Curriculum
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Key Result Area III: Improved livelihood, social protection, and community development.

Strategic challenges	a) Unreliable disability statistics. b) Inaccessible built environment. c) Lack of inclusive and accessible work environment d) Inadequate social protection interventions e) Lack of resources to enhance the capacity of some Community groups. f) Use of proximity mean test as a criteria for social protection interventions. g) Limited access to inclusive vocational training and adapted curricula. h) Lack of trained instructors with skills to teach persons with disabilities. i) Inadequate disaster risk response mechanisms for persons with disabilities. j) Inadequate social protection policies and programmes in place to support caregivers and children of persons with disabilities. k) Exclusion from accessing credits and loan services l) Limited access to credit and loans for business start-up support. m) Limited entrepreneurial skills and mentorship opportunities. n) Workplace discrimination and stigma against persons with disabilities. o) Inaccessible workplaces and lack of reasonable accommodation. p) Inadequate coverage of social protection schemes for persons with disabilities. q) Complex registration procedures to access social protection that exclude many.
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Strategic Objective: To enhance the economic independence, security, and social inclusion of persons with disabilities by ensuring access to skills development, self-employment opportunities, decent wage employment, inclusive financial services, and comprehensive social protection measures.

Strategies	a) Develop monographs on the seven disability domains. b) Enforcement of existing legislation on accessibility to build environment. c) Provision of reasonable accommodation in the workplace. d) Improving access to assistive devices and technologies. e) Develop a national policy framework for assistive technology. f) Community empowerment through the formation of Community groups and organisations of and for persons with disabilities g) Pilot universal disability cash transfer h) Establish targeted financial inclusion programs (grants, microfinance, and credit schemes) for entrepreneurs with disabilities. i) Provide entrepreneurship training, mentorship, and business incubation support tailored to persons with disabilities. j) Expand disability-sensitive social protection schemes (cash transfers, insurance, and pensions). k) Simplify enrolment procedures and provide outreach to ensure persons with disabilities access available benefits. l) Enforce anti-discrimination laws, awareness campaigns, and incentives for inclusive hiring. m) Promote workplace accessibility (assistive technology, adapted infrastructure) and accommodation policies.
Key Result Area IV: Social Inclusion.	
Strategic challenges	a) Lack of nationally agreed definitions, standards and procedures on caregivers. b) Scarcity of programmes that provide training and support for caregivers. c) Negative attitudes, perceptions, and practices that reinforce stigma, prejudice, and discrimination towards persons with disabilities and their families. d) Low and poor self-esteem by persons with disabilities. e) Physical barriers to social participation such as inaccessible transport and public buildings (recreation/sports/parks). f) Lack of awareness and understanding of the specific needs of persons with disabilities by law enforcers and other stakeholders. g) Physical barriers to access justice such as to police stations and courts. h) Lack of knowledge and access to legal representation
Strategies	a) Promote and support access to personal assistance for persons with disabilities. b) Development of Social Protection policies and programmes for caregivers. c) Support persons with disabilities to have fulfilling relationships with members of their families and communities. d) Promoting the use of culture and art for social change. e) Facilitate persons with disabilities to participate in local recreation, leisure, and sports. f) Promote awareness of the rights of persons with disabilities. g) Strengthen support networks.

Key Result Area V: Empowerment of persons with disabilities and their families.

Strategic challenges	a) Existence of attitudinal, institutional, cultural, service access, policy, and physical barriers in the community. b) Lack of diverse and accessible communication channels in some regions/areas. c) Difficulty in the establishment of Community groups in an urban setting due to the existing engagement structures. d) Community participation barriers that include time constraints, cultural limitations, family responsibilities, work responsibilities and low self-esteem. e) Low participation in politics due to non-inclusion in the national discussions and dialogues.
Strategic Objective: To empower persons with disabilities through advocacy and communication, community mobilization, political participation, Community groups and organizations of persons with disabilities as sustainable platforms for leadership, self- representation, and inclusive development.	
Strategies	a) Develop awareness-raising publications and programmes that would change the public's perception on persons with disabilities. b) Use of alternative communication methods to disseminate information to persons with disabilities and the community. c) Advocate through trainings of persons with disabilities on the importance of establishment and participation in Community groups. d) Use existing community gathering structures and forums to engage and discuss community issues. e) Educate persons with disabilities on their rights to be involved through active participation in politics f) Promote representation of persons with disabilities in governance structures and process. g) Strengthen and resource organisations for persons with disabilities.

CHAPTER 02

2.1 Situation Analysis

A scan of the external and internal environments is presented in the strategy using three tools; Strength, Weaknesses, Opportunities and Threats (SWOT) Analysis; Policy, Political, Economic, Social, Technological, Environmental and Legal (PESTEL) Analysis; and Stakeholder Analysis.

2.2 SWOT Analysis

This section outlines the current strengths, weakness, opportunities and possible threats towards implementation of the Community Based Inclusive Development programme.

Strengths ◦Political goodwill ◦Supportive environment by Non-Governmental Organizations, Development Partners and Private Sector ◦Existing Legislations ◦Funding from international community. ◦Strong Government commitment and multisectoral collaboration ◦Existence of a Council for persons with disabilities	Weaknesses ◦Inadequate human and financial resources to implement policy directives. ◦Lack of systematic Inter-governmental coordination and communication ◦Budget cuts from government ◦Lack of disaggregated data for persons with disabilities ◦Unfulfilled financial commitments by some partners. ◦Inadequate coverage of social protection programmes ◦Duplication of roles and responsibilities
Opportunities ◦Partnerships with development partners ◦Affirmative action ◦Fundraising through the National Budgeting Process ◦Support received from sources alternative to government funding.	Threats ◦Change of Government priorities ◦Political Instability ◦Humanitarian emergencies leading to redirection of funds

2.3 PESTEL Analysis

This section outlines the current Policy, Political, Economic, Social, Technological, Environmental and Legal (PESTEL) Analysis towards implementation of the Community Based /Inclusive Development programme.

CATEGORY	ISSUES	DESCRIPTION
Policy	Coordination and reporting	To ensure coordination of State and Non state actors on the implementation of CBID programs.
Political	Regional Politics	Regional and global political stability
Economic	Budget allocation	Allocation of resources through the National Budgeting Process
	Affirmative Action Programmes	Affirmative action measures in procurement of national and county government goods and services by implementing preferential procurement for persons or entities managed by persons with disabilities.
		Where an employer has at least twenty employees, reserve at least five per cent direct employment opportunities for persons with disabilities to secure employment

Social	Governance	Build capacity of right holders and duty bearers on inclusive data for improved programming and service delivery
	Advocacy	To engage media and other entities to sensitize the public on the rights of persons with disabilities and inclusive data.
Technological	Existence of Management information systems	To ensure service delivery is effective and efficient such Community Development Management Information System (CDMIS), Enhanced Single Registry (ESR), Consolidated Cash Transfer Programme Management Information System (CCTPMIS), Children Protection Management Information System (CPMIS)
Environmental	Emergencies of floods, drought, clashes, and fire outbreaks	Delay or disruption of programmes implementation or increased implementation cost
	Climate Change	Increased vulnerability diversion of funding to mitigate the effects
Legal	Existence of the Convention on the Rights of Persons with Disability	To promote, protect and ensure the full and equal enjoyment of all human rights and fundamental freedoms by all persons with disabilities, and to promote respect for their inherent dignity.
	Existence of the Constitution of Kenya	Article 54 recognizes persons with disabilities and their rights.
	Existence of the Persons with Disabilities Act No 4 2025	Provides for the rights and rehabilitation; to achieve equalization of opportunities of persons with disabilities and establish NCPWD.
	The existence of laws on employment	Employment Act 2007, Community Group Registration Act of 2022, The Constitution of Kenya Article 27, Social Protection Policy of 2024, National Social Security Fund Act No 25 of 2013, Protocol to the African Charter on human and people's rights on the Rights of Persons with Disabilities in Africa, Work Injury Benefits Act 2007, Kenya vision 2030.
	The existence of laws on health	UNCRPD Act Article (20,25,26) Protocol to the African Charter on Human and People Rights on the Rights of Persons with Disabilities in Africa 17 and 18; Constitution of Kenya, Chapter 4, Article 43(1, a); Children's Act 2022 Section 20; Mental Health Act 2012.

	Existence of laws on Education	Universal Declaration of Human Rights (1948); Convention on Rights of Persons with Disability Article 24 ◦ United Nations Convention on the Rights of the Child Article 26(2) ◦ The Protocol to the African Charter on Human and People's rights of Persons with Disabilities Article 16 ◦ Kenya Constitution, Articles 43, 53, 54(b) ◦ Basic Education Act 2013(4) Section (28) ◦ Early Childhood Act 2021 ◦ Persons With Disabilities Act 2025 No 4 Section 20 ◦ Children Act no. 29 2022 Section 20 ◦ TVET Act 29 of 2013(iii) Section (5)(7) ◦ Sector Policy for Learners and Trainees with Disabilities 2018
	Existence of laws on economic empowerment	CGR Act 2022 Section 11, Persons with Disabilities Act 2025 Section 4(p), 5(e) PPADA 2015 Section 155

2.4 Management structures in CBID programmes

No.	ACTORS	ROLES
1	Persons with disabilities and their families.	Oversee the implementation of available opportunities. Complementing implementation of home-based rehabilitation, and adaptation of home environment, formation into an OPDs/ Community Group, work as a collaborator (volunteer); Ensure that persons with disabilities are registered and access available CBID Programmes
2	Organizations of Person with Disabilities (OPDs)	Referral and follow-up, data collection and reporting, awareness raising, motivation, advocacy with persons with disabilities, family and community, establishing linkages with other sectors. monitoring implementation of CBID
3	Communities	Management and monitoring of CBID programs
4	Community Social Development Promoters	Ensure that available rehabilitation and habilitation services are utilized by members of the community. Ensure that rehabilitation and habilitation services are available in their community for free or at affordable rates. Advocate for funding of CBID Programmes from both national and county governments.

5	Social Development Committee	Ensure annual surveys are conducted concerning the needs of persons with disabilities. Resource mobilize to assist Community groups in their region. Identify and document all existing and proposed social development projects in their region. Offer advisory opinions on social development projects to ensure diversity and inclusion. Mobilize persons with disabilities to form Special interest groups.
6	Vocational Rehabilitation Committee	Ensure procurement of appropriate assistive technology and equipment in their region's Vocational rehabilitation centers. Ensure increased enrollment of persons with disability in the various vocational rehabilitation centers Link VRC trainees to economic empowerment opportunities
7	Governments (County and National)	Enhance funding towards CBID Programmes, Develop, review and implementation of Policy.
8	Public Benefit organizations (Local, Regional, National, and International Organizations)	Collaborate with National and County Governments in development and implementation of CBID Programmes.
9	The Private Sector	Put in place adequate mechanisms that would ensure easy access to CBID Programmes for their employees. Support CBID Programmes during their corporate responsibility activities.
10	Ministerial Inter Agency Coordinating and Advisory Committee (IACAC)	Monitor CBID Programmes to prevent duplication and promote collaboration between different MCDAs involved in the implementation of the programmes.

CHAPTER 03

3.1 ACCESSIBLE HEALTH SERVICES

The United Nations Convention on the Rights of Persons with Disabilities (CRPD) addresses the right to health for persons with disabilities. Article 25 requires State Parties to “recognize that persons with disabilities have the right to the enjoyment of the highest attainable standard of health without discrimination” and, together this obligation is reinforced by Articles 20 (accessibility) and 26 (habilitation and rehabilitation), which outline measures State Parties should undertake to ensure that persons with disabilities can access gender-sensitive health services, including health-related rehabilitation.

The right to health without discrimination is captured in various international instruments. The Constitution of the World Health Organization (WHO), stipulates “Enjoyment of the highest attainable standard of health is one of the fundamental rights of every human being without distinction of race, religion, and political belief, economic or social condition”. Sustainable Development goal 3 is to Achieve Universal Health Coverage, including financial risk protection, access to quality essential healthcare services, and access to safe, effective, quality, and affordable essential medicines and vaccines for all.

The Constitution of Kenya Chapter 4, Article 43(1, a) and Vision 2030 development blueprints require the country to provide the highest attainable standards of healthcare to all her population. Health also contains freedoms and entitlements. These freedoms include the right to be free from nonconsensual medical treatment such as experiments and research and the right to be free from torture or other cruel, inhuman, or degrading treatments. The health-related entitlements include the right to a system of health protection; the right to prevention, treatment, and control of diseases; access to essential medicines; and participation in health-related decision-making.

Despite these commitments, persons with disabilities often experience poorer health outcomes than the general population and face various barriers to the enjoyment of their right to health. The right to health is not only about access to health services; it is also about access to the underlying determinants of health, such as safe drinking water, adequate sanitation and housing delivered in a manner that respects dignity and promotes inclusion.

3.1.1 Role of CBID in Health

Community-Based Inclusive Development (CBID) plays a critical role in ensuring the right to health for persons with disabilities. CBID works in partnership with the health sector, families, and communities to guarantee that disability-inclusive services are integrated into all levels of health provision as captured in the Persons with Disabilities Act Section. 24(4) of 2025 Section 20(a-g) on health where the Council shall be represented in the implementation of the national health programme under the Ministry responsible for health, in areas of health promotion, prevention, medical care, rehabilitation and assistive devices. CBID also needs to empower persons with disabilities and their families to facilitate their access to health services and to work with other sectors to ensure that all aspects of health are addressed.

Desired Outcome

- i. Persons with disabilities and their family members have improved knowledge about their health and are empowered as active participants in making decisions about their health
- ii. Health systems and workers are aware and provide services without discrimination based on disability, gender, age, or any other factor.
- iii. Persons with disabilities and their family members have access to healthcare and rehabilitation services, preferably in or close to their communities and at an affordable cost.

- iv. Health interventions promote independence, inclusion, and full participation of persons with disabilities in community life.
- v. There is improved collaboration across all development sectors, including education, livelihood, and social sectors, to achieve equitable health outcomes for persons with disabilities.

3.1.2 Elements in the Health Component

A) Health promotion

Health Promotion is the process of enabling people to increase control over the determinants of health and thereby improve their health. The goal of health promotion practice is to provide and maintain conditions that make it possible for persons with disabilities to make healthy choices and facilitate environmental conditions that support healthy behaviors. Health promotion represents a comprehensive social and political process, which embraces actions directed at strengthening the skills and capabilities of individuals and actions directed towards changing social, environmental and economic conditions to alleviate their impact on public and individual health.

Health promotion is a rapidly developing public health field whose emergence has been influenced primarily by a better appreciation of the role of diverse factors that influence persons with disabilities health. These factors are commonly referred to as the determinants of health. They include biological, socioeconomic, environmental, and behavioral elements, and can exert either a positive or negative impact on people's health.

Experiences from within Africa and beyond have shown that health promotion is effective in enabling persons with disabilities, individuals and groups to improve their well-being. It has also been shown that there are now more players in health than was the case before and many of them are to be found outside the health sector.

For health to be created, improved or maintained, supportive conditions must exist. Health development is usually assumed to be the business of health systems, but there is increasing evidence that it is made easier or possible primarily through the creation or improvement of various social, economic and physical conditions and circumstances. Health promotion practitioners therefore need to work with their counterparts in sectors such as agriculture, transport, law enforcement, finance, gender, and education to promote the production of safer and healthier goods and services, provision of healthier public and private services, as well as leaner, more user-friendly environments especially to persons with disabilities.

Health promotion aims to increase control over health and its determinants. The wide range of strategies and interventions available are directed at strengthening the skills of individuals and changing social, economic, and environmental conditions to alleviate their impacts on health.

OUTCOME 1:

The health potential of persons with disabilities and their families is recognized and they are empowered to enhance and/or maintain existing levels of health

B) Prevention

Prevention in health care is to stop health conditions from occurring. Prevention of health conditions (e.g., diseases, disorders, injuries) involves primary prevention (avoidance), secondary prevention (early detection and early treatment) and tertiary prevention (rehabilitation) measures. The focus of this element is mainly on primary prevention as articulated in the Kenya Health Act 2017.

It is estimated that, through better use of primary prevention and health promotion, the burden of disease could be reduced. Even so, it is commonly believed that prevention (as for health promotion) has little, if any, role in the management of health for persons with disabilities.

Health care for persons with disabilities usually focuses on specialized medical care and rehabilitation. However, as previously mentioned persons with disabilities are at risk of other health conditions and at risk of secondary conditions resulting from their primary health conditions.

Just like health promotion, prevention requires the involvement of many different sectors. Within the health sector, primary health care plays an important role and since CBID programmes are most closely linked with primary health care, they can play a significant role in promoting and supporting preventive health care for persons with disabilities.

OUTCOME 2 :

Persons with disabilities are less likely to develop health conditions, related or unrelated to their impairments, that affect their functioning and overall health and well-being; and family members and other community members are less likely to develop health conditions and impairments associated with disability.

C) Medical care

Medical care involves the early identification, assessment and treatment of health conditions and their resulting impairments, with the aim of curing or limiting their impacts on individuals. Medical care can take place at the primary, secondary, or tertiary level of the healthcare system.

Health care personnel often refer persons with disabilities to rehabilitation services for general medical care instead of treating them at primary health care facilities. This is because they lack the awareness that, like the general population, persons with disabilities may acquire a general health condition at any stage throughout their life for which they will need medical care, particularly primary health care. For example, medical care may be needed for respiratory infections, influenza, high blood pressure, middle ear infections, diabetes, tuberculosis or malaria.

Health care personnel have an important role to play in the early identification of conditions that can lead to impairments. It is important that all health conditions are identified and treated early. Some health conditions, if left untreated or uncontrolled, can lead to new impairments or exacerbate existing impairments to persons with disabilities. Early intervention is less traumatic, cost-effective and produces better outcomes.

Majority of persons with disabilities also have specific medical care needs for limited or lifelong periods of time, e.g., persons with epilepsy or people with mental health problems may require drug regimens over a long period of time. Some persons with disabilities may also require surgery to address their impairments.

The Standard Rules on the Equalization of Opportunities for persons with disabilities also outline a list of responsibilities for medical care as a precondition for equal participation in all life activities as articulated in the constitution of Kenya Article 43(2) A person shall not be denied emergency medical treatment and in Health Act 2017 Part 1 Section 5. (1) Every person has the right to the highest standard of health which shall include progressive access for the provision of promotive, preventive, curative, palliative and rehabilitative services.

OUTCOME 3 :

Persons with disabilities access medical care, both general and specialized, based on their individual needs.

D) Rehabilitation

Rehabilitation is a set of measures which enable persons with disabilities to achieve and maintain optimal functioning, independence and participation in their communities. It is relevant both for those who acquire disabilities during their lifetime and for those who have disabilities from birth. Rehabilitation services range from basic to specialized and are provided in many different locations for example, hospitals, homes, and community environments.

Rehabilitation requires the involvement of all development sectors including health, education, livelihood and social welfare. This element focuses on those measures to improve functioning that are offered within the health sector.

Rehabilitation is relevant to persons with disabilities from a broad range of health conditions. Habilitation aims to assist those individuals who acquire disabilities congenitally or in early childhood who have not had the opportunity to learn how to function without them. Rehabilitation aims to assist those who experience a loss in function because of a disease or injury and need to relearn how to perform daily activities to regain maximal function. In the Kenya Health Act 2017 Part XVII on transitional and miscellaneous provisions Section 112(h) makes provision for the Cabinet Secretary in consultation with the Director General to make regulations on Rehabilitation.

OUTCOME 4 :

Persons with disabilities have access to rehabilitation services which contribute to their overall well-being, inclusion and participation.

E) Assistive devices and technologies

A device that has been designed, made, or adapted to maintain or improve an individual functioning and independence is known as an assistive device. Many persons with disabilities benefit from the use of one or more assistive devices. Some common types of assistive devices are mobility devices (e.g., walking sticks, wheelchairs), prostheses (e.g., artificial legs), orthoses (e.g., hand splint), visual devices (e.g., glasses, white canes) and hearing devices (hearing aids).

Access to assistive devices is essential for many persons with disabilities and is an important part of any development strategy. Without assistive devices, persons with disabilities will not participate fully in the society, including in the educational, social and economic aspects of development. They will miss out on education or work, and this perpetuates poor livelihood. Increasingly, the benefits of assistive devices are also being recognized for older persons as a health promotion and prevention strategy.

Persons with disabilities have a right to access assistive devices as articulated in the Persons with Disabilities Act No. 4 2025. Section 4(b) obligates the National Government to undertake investigations, surveys, and research into the causes and nature of disabilities and development of new assistive devices.

OUTCOME 5 :

Persons with disabilities have access to appropriate assistive devices and technologies that are of good quality and enable them to participate fully family, work and in the community.

3.2 INCLUSIVE EDUCATION AND TRAINING

Education is about all people being able to learn what they need and want throughout their lives, according to their potential. It includes “learning to know, to do, to live together and to be” Education takes place in the family, the community, schools, and institutions, and in society.

Kenya as a country faces various barriers to inclusive education such as poverty, marginalization, discrimination, implementation of inclusive policies, limited availability of Inclusive programs, inadequate training for educators, transportation and physical accessibility challenges, stigma and discrimination, limited access to assistive technologies and lack of support services.

Various laws exist, globally and locally, the universal right to education is firmly established in international instruments that have global endorsement such as in the Universal Declaration of Human Rights, Article 26 (2), and the Convention on the Rights of the Child, Article 28 (3).

Article 4 of the Convention on the Rights of persons with disabilities reaffirms the rights discussed above and is the first legally binding instrument to stipulate specifically the right to inclusive education; where State Parties are obligated to ensure an inclusive education system at all levels. The convention further stipulates under Article 24, that States Parties should also ensure that effective individualized support measures are provided in environments that maximize academic and social development, consistent with the goal of full inclusion. This includes the learning of braille, sign language, various modes, means and formats of communication, and orientation and mobility skills.

The Protocol to the African Charter on Human and Peoples on the Rights of Persons with Disabilities under Article 16 stipulates that every person with disability has the right to education and that State Parties shall take reasonable, appropriate and effective measures to ensure inclusive and quality education and skills training for persons with disabilities is realized.

It is estimated by the United Nations Educational, Scientific and Cultural Organization (UNESCO) that over 90% of children with disabilities in low-income countries do not attend school; and an estimated 30% of the world's street children have disabilities. For adults with disabilities, the literacy rate is as low as 3%, and as low as 1% for women with disabilities in some countries.

From these figures, it is evident that steps must be taken to ensure access to education for all children with disabilities. The Kenya Vision 2030 social pillar aims to improve education and training to create a just and cohesive society. these apply equally to children with disabilities, and therefore community-based inclusive development ought to contribute to achieving this target.

Role of CBID in Education

The role of CBID is to work with the education sector to help make education inclusive at all levels and to facilitate access to education and lifelong learning for persons with disabilities.

Desired Outcome

- i. All persons with disabilities have access to learning and resources that meet their needs and respect their rights.
- ii. Local schools take in all children, including children with disabilities, so they can learn and play alongside their peers.
- iii. Local schools are accessible and welcoming; they have a flexible curriculum, teachers who are trained and supported, good links with families and the community, and adequate water and sanitation

facilities.

- iv. Persons with disabilities are involved in education as role models, decision-makers, and contributors.
- v. Home environments encourage and support learning.
- vi. Communities are aware that persons with disabilities can learn and provide support and encouragement.
- vii. There is good collaboration between the health, education, social and other sectors.
- viii. There is systematic advocacy at all levels to make national policies comprehensive to facilitate inclusive education.

3.2.1 Elements in the Education Component

A) Early childhood care and education

Early childhood covers the period from birth to the age of eight years. Expanding early childhood care and education is one of the six education for All goals. Early childhood care and education include a broad range of activities and provisions. Because many aspects of childhood care are covered in the health component, this element mainly focuses on early childhood education and incorporates early intervention and preschool/kindergarten provision. Education in early childhood is important because it influences development. The human brain grows particularly rapidly during the first three years of life, and if adequate stimulation is not received during this period, development will be delayed, sometimes permanently. The early years provide a “window of opportunity” to lay the foundations for the healthy development of language, social ability, thinking and physical skills.

Early Childhood Education sows the seeds for an inclusive society because it is where children with or without disabilities can learn, play and grow together. Early childhood education also increases a child's chances of completing basic education and finding a way out of poverty and disadvantage.

As Early Childhood Education is generally non-compulsory, it is more flexible than primary education and offers an excellent chance to work with a variety of stakeholders, government, public benefit organizations, the private sector, and faith-based organizations.

While Kenya has made progress in access to education, Early Childhood Education for children with disabilities faces various challenges which include, but are not limited to, implementation of inclusive policies, limited availability of Inclusive Programs, inadequate training for Early Childhood Educators, transportation and Physical Accessibility Challenges, Stigma and discrimination, limited access to assistive technologies and lack of support services.

OUTCOME 1 :

All children with disabilities have the best possible start in life and are supported throughout their development in inclusive learning environments.

B) Primary Education

Primary education usually begins around the age of six or seven years and continues into the early teens. Primary education is the gateway to higher levels of education and is a high priority in the context of development.

Primary education must be inclusive and accessible to all. No provider of public education can discriminate on the grounds of gender, ethnicity, language, religion, opinion, disability, or social and economic status.

Primary education is a fundamental right, and the Convention on the Rights of persons with disabilities,

Article 24, states that “... children with disabilities are not excluded from free and compulsory education...” (4). Primary education should be inclusive, provide quality education and equal access, and be available in the local community for children with disability.

At the primary level, barriers to inclusive learning include poverty, stigma to learners with disabilities, many primary schools not adequately equipped to accommodate children with disabilities due to lack of accessible infrastructure, appropriate learning materials, inadequate teachers with the necessary skills to effectively teach children with disabilities and provision of assistive devices.

OUTCOME 2 :

A welcoming, inclusive primary education system, with local schools at the heart of educational activities, exists within the community.

C)Secondary and higher education

In many poor communities, only a few children have access to secondary and higher education, i.e., education beyond the compulsory level, and students with disabilities are either totally excluded or face constant battles to prove their abilities.

The right to secondary and tertiary education for students with disabilities is highlighted in the Convention on the Rights of Persons with Disabilities (Article 24, paragraphs 2(a), 2(b) and 5) (4). Secondary and higher education includes academic programmes and technical/vocational education. despite difficulties and prejudice, there are now examples of students with disabilities, including intellectual impairments, engaging in higher education according to their interests and abilities.

The more disadvantaged a person is, the more they may need access to education beyond the basic level to find employment and full inclusion in society. This is because the most excluded and marginalized often need to demonstrate more skills, knowledge and qualifications than others to attain the same level of survival, employment and inclusion. for students with disabilities, secondary and higher education may be the most important gateway to a full and productive life.

In several countries, it is now a legal requirement for institutions to become accessible, to make “reasonable accommodations”, to offer learning support, and to adapt the curriculum and assessment procedures to ensure that persons with disabilities can access secondary and higher education.

Barriers to inclusive secondary and tertiary education begin with progression from the lower levels of education where many learners with disabilities do not score well in primary schools due to the learning environment. Others include: - lack of necessary infrastructure, staff with necessary skills, resources to accommodate diverse learning needs, physical Accessibility barriers, stigma and discrimination, financial barriers and limited access to assistive t

OUTCOME 3 :

Students with disabilities have opportunities to learn with others and gain qualification, skills and experiences, facilitating their livelihood opportunities, empowerment, and inclusion.

D)Non-formal Education

Non-formal education refers to education that occurs outside the formal school system. Non-formal education is often used interchangeably with terms such as community education, adult education, lifelong education, and second-chance education. It refers to a wide range of educational initiatives in the community, ranging from home-based learning to government schemes and community initiatives. It includes accredited courses run by well-established institutions as well as locally based operations with

little funding.

As non-formal education is diverse, this element has many aspects in common with other elements, particularly lifelong learning. For these guidelines, this element focuses on non-formal education for children and young people outside the regular school system. However, CBID personnel need to be aware that non-formal education reinforces marginalization and stigmatization, so, if possible, it should not be offered as the only educational option for children with disabilities. Inclusion in a regular school should be prioritized as every child's right.

While non-formal education is often considered a second-best option to formal education, it should be noted that it can provide higher-quality education than that available in formal schools. Non-formal education can be preparatory, supplementary or an excellent alternative (where necessary) to formal schooling for all children.

OUTCOME 4 :

Persons with disabilities develop knowledge and skills, which help to improve their quality of life.

E) Lifelong learning

The right to lifelong learning is included in Article 24 of the Convention on the Rights of persons with disabilities. Lifelong learning refers to all purposeful learning activities undertaken on an ongoing basis throughout a person's life, with the aim of improving knowledge, skills, and competencies. Learning is no longer understood to be divided into a time and place for gathering knowledge (and a time and place to apply the knowledge). Without lifelong learning, the capacity of individuals and communities to renew their knowledge and skills and adapt to political, economic, environmental, technological, or social changes is considerably reduced.

Lifelong learning can take place in a variety of settings, both formal and non-formal. It is complementary to other forms of education and continues even when formal education or training opportunities are available. As lifelong learning is comprehensive, this element has many aspects in common with other elements, particularly non-formal education. For the purposes of these guidelines, this element focuses on lifelong learning opportunities for youth and adults with disabilities which exist outside the regular school system.

OUTCOME 5 :

Youth and adults with disabilities have access to quality lifelong learning opportunities and to a variety of learning modes.

3.3 IMPROVED LIVELIHOOD, SOCIAL PROTECTION, AND COMMUNITY DEVELOPMENT.

Access to livelihood and social protection is a fundamental human right for all people, including persons with disabilities. Article 27 of the Convention on the Rights of Persons with Disabilities (CRPD) recognizes the right to work, while Article 28 guarantees the right to an adequate standard of living and social protection. The Constitution of Kenya, Article 43, further affirms every person's right to social security and economic rights.

The Convention requires that the States Parties recognize the Right of persons with disabilities to social protection including an adequate standard of living for themselves and their families, adequate food, housing, and to the continuous improvement of living conditions, and shall take appropriate steps to safeguard and promote the realization of this right without discrimination based on disability.

The Kenya Social Protection Policy (2023) defines social protection as a set of policies, programmes, interventions, and legislative measures aimed at cushioning all Kenyans against poverty, vulnerability, exclusion, risks, contingencies, and shocks throughout their life cycle, and promoting the realization of economic and social rights.

The Constitution of Kenya in the Bill of Rights, guarantees all Kenyans Economic, Social and Cultural rights. Article 43(1)(e) and 43(3) state that “every person has the right to social security” and that “the state shall provide appropriate social security to persons who are unable to support themselves and their dependents.

The Social pillar in Kenya’s vision 2030 aims to improve the quality of life for all Kenyans by targeting a cross-section of human and social welfare projects and programmes. This pillar also makes special provisions for Kenyans with various disabilities and marginalized communities

Despite these legal guarantees and commitments, persons with disabilities continue to face systemic barriers across their life cycle. Children with disabilities are often denied education, youth encounter barriers to training, and adults experience exclusion from decent work. These barriers, coupled with negative stereotypes, perpetuate poverty and dependency. Women with disabilities face multiple and intersecting forms of discrimination in accessing work and resources.

Persons with disabilities in low-income countries are affected by the same factors which cause poverty for others but also face added disadvantages. for instance, Children with disabilities face barriers to education; youth with disabilities face barriers to training while adults with disabilities face barriers to decent work. unfortunately, there is the misconception that Persons with disabilities are incapable of learning skills and working.

By encouraging and facilitating work by women and men with disabilities, community-based inclusive development programmes can help individuals and their families to secure the necessities of life and improve their economic and social situations. By taking into consideration the needs and views of persons with disabilities and making provision for their inclusion in national poverty reduction and other development programmes, opportunities for education, skills acquisition and work can be provided for persons with disabilities and their families, enabling them to emerge from poverty. Accessing livelihood opportunities is one of the key factors in eliminating poverty.

3.3.1 Role of CBID in Livelihood

CBID plays a key role to ensure equal opportunities for persons with disabilities and their families to acquire skills, secure livelihood opportunities, enhanced participation in community life and self-fulfillment. This includes ensuring equal access to training, employment, and financial services, supporting entrepreneurship and self-employment, strengthening social protection to address disability-related costs, and promoting community participation so that persons with disabilities and their families can live with dignity and contribute meaningfully to national development.

Desired Outcome

- i. Persons with disabilities and their families have access to skills development and lifelong learning opportunities.
- ii. Parents of children with disabilities advocate for access to education, skills acquisition, and work opportunities for their children.
- iii. Persons with disabilities have access to decent work opportunities without discrimination in a safe and non-exploitative environment.
- iv. Persons with disabilities have access to credit facilities and business support

- v. Women with disabilities enjoy equal opportunities in work, employment, and access to financial services
- vi. Families of persons with disabilities, especially children and persons with severe disabilities, have access to better means of livelihood.
- vii. All poverty reduction strategies and programmes are inclusive of and include benefit to persons with disabilities and their families.
- viii. The work of persons with disabilities is recognized and valued by employers and community members.
- ix. Local Authorities adopt and apply policies and measures to improve the access to work for persons with disabilities.
- x. Persons with disabilities have access to social protection measures as a right.

Elements in the Livelihood Component

A) Skills development

Skills are essential for work. There are four main types of skills: foundation skills, technical and professional skills, business management skills and core life skills. These skills can be acquired through traditional home-based activities and education, in mainstream vocational training centers and as an apprentice with members of the community. A combination of all four types of skills ensures greater success in finding decent work and earning an income. CBID programmes need to identify and promote opportunities for persons with disabilities to acquire all four types of skills.

These skills must be delivered through inclusive schools, vocational training centers, apprenticeships, and community-based initiatives, in line with the ILO Human Resource Development Recommendation (No. 195).

The Kenyan National Skills Development Policy (2023) promotes socio-economic development through the development of a skilled labour workforce that is employable, productive and competitive.

OUTCOME 1 :

Persons with disabilities acquire skills that enhance their employability, entrepreneurship, and participation in community development.

B) Self-development

Article 27 of the Convention on the Rights of Persons with Disabilities affirms the right of persons with disabilities to work on an equal basis with others, including those who acquire disability during employment. In line with this, Section 5(3) of the Kenyan Employment Act (2007) prohibits employers from discriminating or harassing employees on the grounds of disability, whether directly or indirectly.

Self-employment is a vital pathway for persons with disabilities in low-income settings, enabling women and men to earn a livelihood and contribute to the well-being of their families and communities. CBID programmes play a critical role in supporting this right by facilitating access to resources, training, and opportunities to start or grow income-generating activities and small businesses.

OUTCOME 2 :

Persons with disabilities have opportunities to earn their livelihood through self-employment, improve their standard of living and contribute to the well-being of their families and communities.

C) Wage employment

Wage employment refers to any salaried or paid work under contract, whether formal or informal. persons with disabilities continue to face significant barriers in accessing decent wage employment. However, progress is emerging as more businesses embrace inclusive hiring practices. CBID programmes have a key role in removing these barriers and promoting equal access to dignified employment opportunities.

The ILO Minimum Wage Fixing Convention, 1970 (No. 131), requires states to establish minimum wage systems that protect workers from unduly low pay. These systems must be regularly reviewed and adjusted in consultation with social partners, balancing the needs of workers and their families with prevailing economic conditions.

The Regulations of Wages and Conditions of Employment Act and the Regulation of Wages Order subsidiary to Chapter 229 addresses the issues of minimum wage in Kenya. A tradition has been established according to which the Minister of Labour, in exercise of Section 11 of the Regulation of Wages allows the cabinet secretary in charge of labour to order the increment of minimum wages.

OUTCOME 3 :

Persons with disabilities have equal opportunities to access waged employment and earning a living on an equal basis as others.

D) Financial services

Persons with disabilities equally have the same needs for financial services and ought to benefit from micro credit and micro finance services to enable them to be self-reliant and economically empowered.

OUTCOME 4 :

Persons with disabilities and their families have equal access to financial services to support the development of their economic and other activities and improve their standard of living.

E) Social protection

Article 28 of the Convention of rights for Persons with disabilities recognizes the right of persons with disabilities to an adequate standard of living for themselves and their families, including adequate food, clothing and housing, and to the continuous improvement of living conditions, and shall take appropriate steps to safeguard and promote the realization of this right without discrimination based on disability.

The States Parties recognize the right of persons with disabilities to social protection and to the enjoyment of that right without discrimination based on disability, and shall take appropriate steps to safeguard and promote the realization of this right,

Social protection measures are intended to provide a safety net to protect people against extreme poverty and loss or lack of income through illness, disability, or old age. persons with disabilities have an equal right to the social protection measures available to citizens generally.

OUTCOME 5 :

Persons with disabilities who are out of work, who are unable work, or whose resources from occupational activities are not sufficient to maintain their livelihood enjoy a minimum standard of living through social protection measures.

3.4 SOCIAL INCLUSION

Persons with Disabilities face many barriers in the society and therefore often have fewer opportunities to participate in social activities. Being actively included in the social life of one's family and community is important for personal development. The opportunity to participate in social activities has a strong impact on a person's identity, self-esteem, quality of life, and ultimately his/her social status.

Community-Based Inclusive Development programmes have in the past focused on health issues and rehabilitation activities, often ignoring the social needs of persons with disabilities. To date, topics such as relationships, marriage and parenting may be seen as too sensitive or too difficult to address, while access to cultural, sporting and recreational activities, and justice, is seen as unnecessary.

Kenya has provided legislative frameworks to address these challenges and ratified the Protocol on the African Charter on Human and Peoples' Rights on the Rights of Persons with Disabilities in Africa in February 2022 which stipulates under Article 14 that every person with a disability has the right to live in the community with choices on an equal basis with others and that state parties shall take effective and appropriate measures to facilitate full enjoyment by persons with disabilities of the right to live in the community on an equal basis with others.

The social component therefore highlights the importance of these issues in the lives of persons with disabilities and how Kenya through the implementation of this strategy needs to establish CBID programmes to address them.

3.4.1 Role of CBID in Social Component

Persons with disabilities have meaningful social roles and responsibilities within their families and communities and should be treated as equal members of society.

Desired Outcome

- i. Persons with disabilities are valued as members of their families and have a variety of social roles and responsibilities.
- ii. Persons with disabilities and their families are encouraged and supported to contribute their skills and resources to the development of their communities.
- iii. Communities recognize that persons with disabilities are valued members and can make positive contributions to the community.
- iv. Barriers that exclude persons with disabilities and their families from participating in social roles and activities are and should be addressed.
- v. Local government authorities respond to the needs of persons with disabilities and their family members and provide effective social support and services where required.

3.4.2 Elements in the Social Component

A) Personal assistance

In Kenya, personal assistance is normally referred to as caregiving. It includes both the formal assistance mainly offered to employees and informal assistance with family members, friends, neighbors and/or volunteers. Caregivers play a very important role in the lives of persons with disabilities as they aid them in accessing socio-economic, educational, food security and health needs among others.

Caregiving responsibilities sometimes may include help with one or more activities that are important for daily living of the person with a disability such as bathing and dressing, paying bills, shopping, providing transportation or emotional support or helping in managing a chronic disease or disability. In low-income countries where there are limited social services and benefits, families are often the main or only means of this support.

While formal options are often very limited, this element shows how CBID programmes can utilize community resources to ensure persons with disabilities and their families are able to access a range of different options for personal assistance which are appropriate to their individual needs.

The CRPD Article 19 emphasizes that state parties should ensure that persons with disabilities have a choice to live independently as well as who to live within the community and that they have access to quality and affordable support services and assistive devices.

In addition, the Protocol to the African Charter on Human and Peoples' Rights on the Rights of Persons with Disabilities in Africa obligates state parties to ensure persons with disabilities who require intensive support, and their families have adequate and appropriate facilities and services, including caregivers and relief services. The Protocol also makes provision for persons with disabilities to have access to a range of in-home, residential, and other community support services necessary to support living and inclusion in the community and that community-based inclusion services are provided in ways that enhance their participation and inclusion in the community.

OUTCOME 1 :

Persons with disabilities are able to access personal assistance which meets their needs and enables them to lead active and fulfilling lives.

B) Relationships, marriage, and family

Relationships are as important for persons with disabilities as they are for everyone else. This element looks at the ways in which CBID programmes can support persons with disabilities to enjoy a variety of social roles and responsibilities associated with relationships. By working closely with a range of stakeholders in the community, CBID programmes can help to increase awareness about disability, challenge negative family and community attitudes, and prevent and address violence against persons with disabilities.

The Convention on the Rights of Persons with Disabilities Article 23 recognizes persons with disabilities right to marry and found a family, decide freely and responsibly on the number and spacing of their children, have access to age-appropriate information, reproductive and family planning education, and to retain their fertility.

The Constitution of Kenya under Article 45 recognizes the family as the natural and fundamental unit of society and the necessary basis of social order and should therefore enjoy the recognition and protection of the State. It further states that every adult has the right to marry a person of the opposite sex, based on the free consent of the parties. Article 45(3) the Constitution further stipulates that parties are entitled to

equal rights at the time of the marriage, during the marriage and at the dissolution of the marriage. In addition to the Constitution, at the national level, there are various legislations which are relevant to the family in general, to specific family members and issues affecting them.

In addition to the Constitution, at the national level there are various legislations which are relevant to the family in general and to specific family members and issues affecting them, among which are: The Marriage Act 2014, The Matrimonial Property Act 2013, The Protection Against Domestic Violence Act 2015, The Children 's Act 2022, The Social Protection Act, 2025, The Persons with Disabilities Act 2025 and The Sexual Offences Act 2006.

OUTCOME 2 :

Persons with disabilities fully realize their positions and roles within their families and communities.

C) Culture and arts

Participation in cultural and arts activities is important for personal growth and development. It helps to establish personal identity and to provide a sense of belonging as well as opportunities for persons with disabilities to participate in.

Persons with disabilities are being made to participate in cultural practices which are harmful and retrogressive. Some cultural practice perpetuates the abuse of persons with disabilities. This element identifies and addresses barriers that exclude persons with disabilities from participating in the cultural and artistic life of their families and communities. It also explores the role that culture and arts can play in challenging stigma and discrimination directed towards persons with disabilities, and promoting diversity, inclusion, and participation.

The CRPD Article 30 recognizes the right of persons with disabilities to take part on an equal basis with others in cultural life. The Protocol on the African Charter on Human and Peoples' Rights on the Rights of Persons with Disabilities in Africa obligates the states under Article 25 to take effective and appropriate policy, legislative, budgetary, administrative, and other measures to ensure persons with disabilities right to participate in sports, recreation, and cultural activities.

The Constitution of Kenya under Article 11 recognizes culture as the foundation of the nation and as the cumulative civilization of the Kenyan people and nation.

OUTCOME 3 :

Persons with disabilities contribute to and participate in the cultural and artistic lives of their families and communities.

D) Recreation, leisure, and sports

Persons with disabilities have been included in sporting and recreation activities, notably the Paralympic games and special Olympics, including national and international games. Recreational, leisure and sporting activities are important for health and well-being and for strengthening the cohesion of the community.

Persons with disabilities still face several challenges in participating in recreation, leisure and sporting activities for example insufficient equipment that aid sporting activities and where available they are unaffordable to persons with disabilities.

This element looks at the benefits these activities can have for persons with disabilities and provides practical suggestions about how CBID programmes can work with a range of stakeholders to increase opportunities for the inclusion and participation of persons with disabilities.

OUTCOME 4 :

Persons with disabilities participate both actively and as spectators in recreational, leisure and sporting activities on an equal basis with others.

E) Justice

All community members may need to access justice at some point in time. At the local level it is important to be aware of existing laws that can be used to ensure persons with disabilities can access their rights and entitlements as outlined in the Convention on the Rights of Persons with Disabilities.

The processes of accessing justice in Kenya are out of reach for many, more so persons with disabilities. This is due to various factors such as lack of awareness and negative attitudes on the part of personnel in the justice system about the unique nature of the needs of persons with disabilities, inaccessible information and communication formats, the cumbersome and rigorous reporting processes, procedural and evidentiary methods used in the judicial institutions and the cost of litigation which is exorbitant for the majority of persons with disabilities.

The justice element focuses on how CBID programmes can support persons with disabilities to claim their rights by raising their legal awareness and facilitating their access to a range of legal processes to challenge injustices.

Persons with disabilities as consumers of justice are entitled to access a speedy and fair adjudication process as captured under Article 13 of CRPD which obligates state parties to ensure effective access to justice for persons with disabilities. The Protocol on the African Charter on Human and Peoples' Rights on the Rights of Persons with Disabilities in Africa under Articles 7 and 13 further obligates state parties to ensure equal recognition of persons with disabilities before the law and ensure that persons with disabilities have access to justice on an equal basis with others.

The Constitution of Kenya Article 10 defines national values as patriotism, national unity, sharing the rule of law, human dignity, equity, social justice, inclusiveness, equality, human rights, non-discrimination, and protection of the marginalized, transparency and accountability that should be replicated at the family and community level. Additional articles that make reference to persons with disabilities access to justice on equal basis with other under the Constitution include Articles 20, 21, 22, 23, 47, 48, 49, 50, 51 and 54.

OUTCOME 5 :

Persons with disabilities have access to justice on an equal basis with others to ensure full enjoyment and respect of human rights.

3.5 EMPOWERMENT OF PERSONS WITH DISABILITIES AND THEIR FAMILIES

Empowerment helps facilitate the mainstreaming of disability across each sector and ensures that everybody can access their rights and entitlement. Empowerment can be described as having a say and being listened to, self-power, own decision making, having control, or gaining further control, being free, independence, being capable of fighting for one's rights and being recognized and respected as equal citizens and human beings with contributions to make.

The starting point of any CBID program should be to facilitate the empowerment of persons with disabilities and their families and communities as this will lead to achievement of goals, outcomes, and sustainability.

3.5.1 Role of CBID in Empowerment

The role of CBID is to contribute to the empowerment process by promoting, supporting, and facilitating the active involvement of persons with disabilities and their families in issues that affect all aspects of their lives.

Desirable Outcomes

- i. Persons with disabilities can make informed choices and decisions.
- ii. Persons with disabilities are active participants and contributors in their families and communities.
- iii. All forms of barriers in the community are removed and persons with disabilities are accepted as people with potential.
- iv. Persons with disabilities and their families can access all available development benefits and services in their communities.
- v. Persons with disabilities and their families unite to form their own groups and organizations, working together to tackle common issues.

3.5.2 Elements in the Empowerment Component

A) Advocacy and Communication

Self-advocacy and communication are crucial for individuals with disabilities to empower themselves. They connect them to their families and communities, empowering them to make choices and express opinions. Communication plays a significant role in self-advocacy, as it helps them build confidence and self-esteem. Community Based Inclusive development (CBID) programs help improve communication and engagement, ultimately leading to the ability to self-advocate.

OUTCOME 1 :

Persons with disabilities to be able to speak out for themselves.

B) Community Mobilization

It is the process of bringing together as many stakeholders as possible e.g. persons with disabilities, family members, Community groups, disabled people's organizations, community members, local authorities, local leaders, decision- and policymakers, to address barriers within the community and ensure the successful inclusion of persons with disabilities in their communities with equal rights and opportunities to raise people's awareness of and demand for a particular programme, to assist in the delivery of resources and services, and to strengthen community participation for sustainability and self-reliance.

A lot can be achieved when people from different parts of the community share a common goal and actively participate in both identifying needs and being part of the solution. It helps to empower communities and enable them to initiate and control their own development. Community mobilization is a strategy that CBID programmes can use to ensure matters disability are inclusive within the community. It is a strategy that can be used by CBID programmes to engage community stakeholders in CBID activities towards inclusive development.

OUTCOME 2 :

Local communities are empowered to remove barriers for persons with disabilities and their families and play an active role in facilitating the inclusion of persons with disabilities and their families in community activities.

C) Political Participation

Political participation involves various activities where individuals including persons with disabilities can express their opinions on matters affecting all aspects of their lives both globally and nationally. These activities can range from Vying for key political positions, civic participation and voting.

persons with disabilities often face significant barriers affecting their political participation. This lack of participation hinders their voices, rights to equity, and access to health, education, livelihood, and social sectors, thereby limiting their ability to influence change and decision-making.

The policy agenda often neglects the needs of persons with disabilities, particularly in resource-limited areas. This results in limited inclusive mainstreaming programs and disability-specific services, and poor implementation of policies addressing disability, resulting in numerous societal barriers for persons with disabilities.

OUTCOME 3 :

Persons with disabilities participate in political and public life on an equal basis with others.

D) Community groups

In Kenya, the Community Group Registration Act 2022 has defined a community group as a voluntary association of individuals from the same community which is self-organized for a common purpose aimed at improving the livelihood of the group members or for a community benefit and includes special interest groups, community project and community-based organization but shall not include Public Benefit Organization, or groups formed to champion a political cause or contrary to public policy.

Self-Help Groups are informal groups of people who come together to address common problems and undertake socio economic empowerment activities. The groups mainly focus on micro credit programs and income generating activities. these groups are used in the development sector for poverty alleviation, human development, and social empowerment. They focus on microcredit programs and income-generating activities. Self-help groups are a key element of the Community-Based inclusive Development matrix helping to achieve the goals of inclusion and ownership by Persons with disabilities and enhancing their participation in development processes.

OUTCOME 4 :

Persons with disabilities and their family members participate in groups to resolve common problems, enhance their individual strengths and improve quality of life.

E) Organizations of Persons with Disabilities

Organizations of Persons with disabilities are identified as organizations that are formed and led by persons with disabilities. They have a larger scope, advocating for human rights and influencing policy and resource allocation. These organizations have a larger scope and are a global movement of persons with disabilities united to influence decision making in matters affecting them in governments and all sectors of society.

OUTCOME 5 :

CBID programmes and organizations for persons with Disabilities work together to ensure the implementation of the Convention on the Rights of Persons with disabilities and community-based inclusive development.

ANNEX I: IMPLEMENTATION MATRIX FOR 2026-2030

Key Result Area I: Accessible health services

Strategic Objective I: To promote access to inclusive and comprehensive health services to persons with disabilities including rehabilitation, assistive technologies and sexual and reproductive health services, while strengthening promotive and preventive care to uphold the right to the highest attainable standard of health.

Strategy	Activity	Output	Output indicator	Mean of verification	Time frame	Lead agency
Adopt integrated community health approaches that respond to environmental, socio-economic, and political challenges while ensuring full inclusion of persons with disabilities.	Conduct community health assessments that include disability data.	Disability-disaggregated community health reports developed	Number of community health assessments disaggregated by disability	Assessment Reports on health information on persons with disability Health Surveys	2026-2027	Ministry of Health State Department for Social Protection (Directorate of Social Development), NCPWD
	Facilitate the inclusion of persons with disabilities in community self-help groups and social networks as a means to strengthen participation, and peer support, and social inclusion, contributing to improved health, and well-being.	Community Groups Registered in the Community in CDMIS Persons with disabilities actively participating in Community Groups and social networks that enhance their health outcomes.	Number of groups Registered. Number of persons with disabilities participating in the Community Groups	System generated list of registered Community Groups Group membership list Reports including community development office reports	2026-2027	State Department for Social Protection (Directorate of Social Development) Ministry of Interior and National Administration, OPDs, Community Groups
	Integrate disability inclusion into county, national community health strategies including inclusive SHR	Health plans and strategies that reflect disability inclusion	Number of county, national plans integrating disability inclusion.	Approved strategic plans, policy documents	2026-2028	Ministry of Health, CoG, County Governments, OPDs
	Build the capacity of local artisans and community-based technicians to produce, adapt, and repair affordable assistive technology, ensuring sustainable access for persons with disabilities. Train local artisans to make and repair assistive devices for incoming generation	Local pool of trained artisans, technicians providing sustainable assistive technology services.	Number of trained Local artisans and technicians Number of persons with disabilities accessing locally made or repaired Assistive Technologies	Training reports, List of trainees Assistive Technology distribution list Maintenance logs	2026-2027	Ministry of Health State Department for Social Protection (Directorate of Social Development), NCPWD, NFDK

	promote the participation of persons with disabilities in health services decision making spaces, bodies including health boards and committee's county, and national levels	Persons with disabilities engaged in health governance and oversight.	Number of health decision making spaces including boards, committees with persons with disabilities representation.	Decision making spaces: Board, committee membership rosters, Meeting reports.	2026-2028	Ministry of Health, CoG, County Governments
Expand access to preventive and promotive health services, including early identification and intervention, to safeguard the health of persons with disabilities across all stages of life	Implement early childhood screening and intervention programmes.	Early identification and referral systems functional	Number of children screened Number of referrals made for intervention	Screening registers, referral forms health worker reports.	2026-2027	Ministry of Health State Department for Social Protection (Directorate of Social Development) County Governments
	Raise awareness and conduct outreach among families and communities on the importance of disability-inclusive MCH services.	Families informed about MCH services and rights of women, children with disabilities. improved access to MCH services in rural, under-served areas	Number of awareness campaigns conducted Number of community members reached. Including women with disabilities Number of women, children reached through outreach.	Awareness campaign and outreach reports, participant surveys	2026-2028	Ministry of Health County Governments OPDs
	Train healthcare workers in early identification and referral including disability-inclusion in service delivery and inclusive Sexual and Reproductive Health	Healthcare workers equipped with disability-inclusive skills	Number of healthcare workers trained in disability-inclusion in service delivery.	Training reports, Participant lists.	2026-2027	Ministry of Health County Government's
	Facilitate equitable access to preventive and promotive care for women with disabilities and mothers of children with disabilities.	Inclusive community health campaigns conducted Women with disabilities receive inclusive preventive and promotive services.	Coverage among children with disabilities. Number of women with disabilities receiving care	Campaign reports, Registers	2026-2028	Ministry of Health County Government's

	Develop inclusive preventive health guidelines and Integrate disability inclusion into maternal and child health policies, guidelines, and outreach programmes.	Inclusive preventive health guidelines adopted. policies and guidelines revised to be disability-inclusive	Number of guidelines developed, adapted. Number of policies, programmes updated to include disability.	Published guidelines, MoH circulars. Policy documents	2026-2027	Ministry of Health County Government's
Improve accessibility of health facilities and services by removing physical, communication, and systemic barriers to care for persons with disabilities.	Conduct accessibility audits of health facilities, implement accessibility improvements in facilities and provide reasonable accommodation in service delivery	Accessibility gaps identified Health facilities upgraded with universal design features Reasonable accommodation routinely available.	number of facilities with completed accessibility audits. Number of facilities upgraded for accessibility. % Of facilities offering reasonable accommodation.	Audit reports, facility accessibility plans Site inspection reports, photos, procurement records. Facility checklists	2026-2028	Ministry of Health County Government NCPWD
	Engage OPDs and communities in monitoring accessibility of health services to ensure sustainability and accountability	Community, OPD accessibility monitoring mechanisms in established	Number of health facilities reviewed	Monitoring or review reports	2026-2027	Ministry of Health County Government NCPWD OPDs
	Facilitate equitable access to affordable and appropriate assistive devices for persons with disabilities through strengthened supply systems, subsidies, and community-level distribution mechanisms.	Community-level access points and referral systems established for assistive technology Assistive technologies made available to persons with disabilities.	Number of distribution points established Percentage of persons with disabilities receiving assistive technologies	AT Distribution logs reports AT recipients list	2026-2027	State Department for Social Protection (Directorate of Social Development) and Senior Citizen affairs NCPWD NFDK
	Organize health literacy workshops for persons with disabilities and their families and facilitate peer-to-peer health education through OPDs and Community Groups	Persons with disabilities and families trained on health literacy Persons with disabilities empowered to educate peers on health services.	Number of workshops conducted, Number of participants reached. Number of peer educators trained;	Training reports, attendance lists. Activity reports	2026-2027	Ministry of Health OPDs, Community Groups County Governments

	Strengthen referral and follow-up systems for rehabilitation	Functional referral and follow-up systems linking community, primary, and specialized rehabilitation services.	Percentage of facilities with functional referral pathways for rehabilitation. Percentage of persons with disabilities receiving follow-up care after referral.	Referral logs, Follow-up registers, records.	2026-2027	Ministry of Health OPDs, Community Groups County Government
	Improve community-based psychosocial support	Community-based psychosocial networks established and linked with rehabilitation and social inclusion programmes	Number of peer counseling and family support groups established. % Of persons with disabilities accessing community-based psychosocial support Number of linkages created between psychosocial, rehabilitation, and social inclusion programmes.	Community health worker reports, Programme records,	2026-2027	Ministry of Health OPDs, Community Groups County Government
Promote positive community attitudes and perceptions through sustained sensitization and awareness programmes on disability rights and inclusion in health.	Conduct sensitization forums on disability rights and health inclusion.	Communities engaged and sensitized disability and health rights.	Number of forums held annually	Forum reports, attendance registers.	2026-2028	Ministry of Health County Government's OPDs State Department for Social Protection (Directorate of Social Development) NCPWD
	Develop and disseminate inclusive Health promotion IEC (Information, Education, Communication) materials in accessible formats	Accessible IEC materials distributed	Number of IEC materials produced, disseminated.	Copies of IEC materials, distribution records.	2026-2028	Ministry of Health County Government's OPDs State Department for Social Protection (Directorate of Social Development) NCPWD

	Institutionalize disability mainstreaming in all education and training curricula, including teacher training colleges, health worker training, justice system personnel training, and other professional development programmes, to foster inclusive attitudes and practices across society.	Disability inclusion integrated into curricula for across sectors including teachers, justice sector personnel, and health professionals.	Percentage of national curricula revised to include disability mainstreaming. Number of persons trained using the integrated curriculum	Approved curriculum documents	2026-2028	KICD MOH Ministry of Education Judiciary State Department for Social Protection (Directorate of Social Development)
	Engage community-based organizations and OPDs, community leaders to promote inclusive health in communities	Community leaders, OPDs actively promoting inclusion.	Number of leaders, OPDs engaged.	Campaigns reports reports	2026-2028	MOH County Government OPDs, Community
Strengthen inclusive health communication and information systems	Develop, adopt, and implement a national disability-inclusive health communication strategy, in partnership with OPDs, to ensure that health information is accessible, understandable, and responsive to the diverse needs of persons with disabilities.	A disability-inclusive health communication strategy developed, validated, and rolled out.	Approved inclusive health communication strategy. Percentage of health facilities applying the communication strategy guidelines Number of OPDs engaged in the development	Approved strategy document. Stakeholder consultation reports	2026-2028	MOH, County Government OPDs State Department for Social Protection (Directorate of Social Development)
	Establish accessible health information platforms including inclusive feedback and complaints systems in	Accessible platforms (toll-free hotlines, SMS alerts, mobile apps, accessible websites) established and operational. Disability-inclusive feedback and complaints mechanisms functional in facilities	Number of inclusive health information platforms established. Percentage of persons with disabilities reporting access to timely health information. Number of facilities with accessible feedback systems; % of persons with disabilities satisfied with response to complaints.	Platform usage reports, service analytics Facility reports, feedback registers, user surveys.	2026-2028	MOH, State Department for Social Protection (Directorate of Social Development) OPDs, County Government

Key Result Area II: Inclusive Education and training

Strategic Objective II: Strategic Objective: To enhance the economic independence, security, and social inclusion of persons with disabilities by ensuring access to skills development, self-employment opportunities, decent wage employment, inclusive financial services, and comprehensive social protection measures.

Strategy	Activity	Output	Output indicator	Mean of verification	Time frame	Lead agency
Accessible infrastructure and transportation systems.	Community Accessibility audits	Community accessibility Audit report	No. of transport operators trained on disability etiquette	Pre and post test results	2026-2028	Ministry of Transport & Infrastructure, ICT, NCP-WD, State Department for Social Protection (Directorate of Social Development) OPDs, MOH, MOE, County Government Engineers board, PBOs, Private Sector Foundations, UN Agencies, Development Partners, Matatus Owners Association, OPDs, KNCCI
	Inclusive design workshops		No. of persons with disabilities reporting improved access.	Workshop attendance lists and reports		
	Advocacy for accessible transport and infrastructure		No. of local artisans trained on accessible design	Completed audit forms		
	Behavior changes campaigns		No. of community forums held on accessibility.	Survey results		
	Inclusive policy dialogues		No. of public facilities upgraded for accessibility.	Site visits Audit Reports		
Provide financial assistance to families of learners with disabilities which may include subsidizing assistive devices and other school-related expenses	Conduct needs assessments and household mapping	Assistive technology distributed Stipend issued	No. of families receiving stipends	Attendance list and training reports	2026-2027	NCPWD, State Department for Social Protection (Directorate of Social Development), MoE, MoH, County Governments, PBOs, Private Sector Foundations, UN Agencies, Development Partners, National Treasury, KRA, KNBS, MFIs
	Implement subsidy programs for assistive technology		No. of assistive devices subsidized or distributed	Distribution logs		
	Pilot school-linked cash transfer programs			Disbursement Records		
	Launch transport support initiatives					
	Establishing public-private partnerships					

	Provide financial literacy training and empowerment sessions for parents and caregivers	Parents, caregiver financial literacy held	No. of parents and caregivers trained on financial literacy % Increase in school attendance among supported learners	Mobile money statements	2026-2027	NCPWD, State Department for Social Protection (Directorate of Social Development), MoE, MoH, County Governments, PBOs, Private Sector Foundations, UN Agencies, Development Partners, National Treasury, KRA, KNBS, MFIs
Develop an adapted curriculum and offer specialized Support Services, including speech therapy, occupational therapy, and counselling.	Develop and Pilot adapted curriculum in selected schools	Adapted curriculum modules developed	No. of adapted curriculum modules developed	Curriculum drafts, stakeholder review reports	2026-2027	MoE, KICD, KISE, NITA PARENTS, CAREGIVERS State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments, PBOs, Private Sector Foundations, UN Agencies, Development Partner
	Providing speech therapy services to support communication development. Provide counselling and psychosocial support	Speech therapy sessions conducted	No. of speech therapy sessions conducted No. of occupational therapy sessions delivered No. of counselling sessions provided	Therapist logs, caregiver feedback forms	2026-2029	MoE, MoH PARENTS, CAREGIVERS State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments, PBOs, Private Sector Foundations, UN Agencies, Development Partners, Physiotherapy Council of Kenya, Kenya Occupational Therapists Associations.
	Deliver occupational therapy services to enhance learners' functional and motor skills.	Occupational therapy sessions delivered	Number of occupational therapy sessions delivered	Occupational Therapy progress sheets, learner profiles		MoE, MoH Parents, Caregivers State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments, PBOs, Private Sector Foundations, UN Agencies, Development Partners, Physiotherapy Council of Kenya, Kenya Occupational Therapists Associations.

Provide comprehensive training for educators on inclusive teaching practices and strategies for supporting learners with disabilities.	Develop and Deliver a National Training Program on Inclusive Education	A standardized training curriculum and delivery framework developed and implemented.	No. of educators trained using the standardized curriculum. No. of counties covered by the training program.	Training manuals Curriculum documents. Attendance and training completion reports	2026-2028	MoE, KICD, NITA, State Department for Social Protection (Directorate of Social Development), NCPWD, OPDs, PBOs, Development Partners, UN Agencies, Private Sector, KISE, Commission for University Education, County Governments, CoG
	Establish Inclusive Education Resource Hubs in Selected Counties	10 Functional resource centers equipped with assistive technologies and inclusive education materials established	No. of resource hubs established and operational.	Budgetary resource allocation Inventory records of materials and assistive technologies.	2026 - 2028	MoE, KISE, KICD, NITA, State Department for Social Protection (Directorate of Social Development), NCPWD, OPDs, PBOs, Development Partners, UN Agencies, Private Sector, KISE, Commission for University Education, County Governments, CoG
Invest in early intervention programs.	Establish County-Based Early Childhood Screening and Assessment Centres	3 Early identification and referral systems established.	Number of assessment centers established and operational. Number of children screened for developmental delays or disabilities.	Centre establishment reports. Screening and referral records.	2026-2028	MoH, MoE, County Governments, CoG, Private Sector, NCPWD, State Department for Social Protection (Directorate of Social Development), UN Agencies, Kenya Pediatrics Association (KPA), OPDs
	Train Community Health Volunteers (CHVs) and ECDE Teachers on Early Detection and Intervention Techniques	Trained frontline workers (CHVs and ECDE teachers) equipped with skills	Number of CHVs and ECDE teachers trained. Number of children identified early and receiving support services.	Training attendance Reports from CHVs and ECDE teachers on cases identified and referred.	2026-2028	MoH, MoE, County Governments, CoG, Private Sector, NCPWD, State Department for Social Protection (Directorate of Social Development), UN Agencies, Kenya Pediatrics Association (KPA), OPDs
Equitable Assessment and Evaluation of the Adapted Curriculum	Develop Inclusive Assessment Tools Aligned with the Adapted Curriculum	Disability-responsive assessment tools developed	No. of inclusive assessment tools developed	Finalized assessment toolkits and guidelines.	2026-2028	MoH, MoE, County Governments, CoG, Private Sector, NCPWD, State Department for Social Protection (Directorate of Social Development), UN Agencies, OPDs

Key Result Area III: Improved Livelihood, social protection, and community development

Strategic Objective III: To enhance the economic independence, security, and social inclusion of persons with disabilities by ensuring access to skills development, self-employment opportunities, decent wage employment, inclusive financial services, and comprehensive social protection measures.

Strategy	Activity	Output	Output indicator	Mean of verification	Time frame	Lead agency
A nationally recognized repository of disability domain monographs	Draft and publish monographs	Monographs developed	No. of monographs produced	Published monographs, dissemination reports	2026–2027	MoH, State Department for Social Protection (Directorate of Social Development)
A national strategic framework for assistive technology policy	Develop national assistive technology policy	National policy developed	Policy submitted to parliament	Cabinet Memo	2026–2028	MIT, State Department for Social Protection, NCPWD
Reasonable accommodation in workplaces Support employers		Adjustments provided	No. of workplaces adopting	Inspection reports, employer records	2026–2028	State Department for Labour, FKE, KEPSA
Enforce accessibility in built environment	Finalize disability policy	National policy submitted	Policy approved	Cabinet Memo	2026–2028	State Department for State Protection (Directorate of Social Development). Parliament
Access to assistive technology	Provide assistive technology	Tech provided	No. of persons with disabilities receive assistive tech	Product lists	2026–2028	MoH, NCPWD
	Provide assistive devices	Devices issued	No. of persons with disabilities issued with devices	Assessment forms	2026–2028	MoH, PBOs, OPDs
Community empowerment (Community Groups & OPDs)	Provide start-up capital	Groups supported	No. of Community Groups supported	Reports	2026–2028	State Department for Gender, County Government
	Train OPDs on rights	OPDs empowered	No. of OPDs trained	Training reports	2026–2028	NCPWD, OPDs
	Build Community Groups on business dev.	Community Groups trained	No. of groups trained	Training reports	2026–2028	State Department for Trade, PBOs
Pilot disability cash transfer	Pilot in 2 counties	Cash transfer piloted	No. of pilots completed	Pilot reports	2026–2026	State Department for National Treasury, State Department for Social Protection
Targeted financial inclusion programs	Develop disability-friendly financial products	Accessible products created	No. of products developed	Product docs, agreements	2026–2026	National Treasury, MFIs
	Provide grants, credit to persons with disabilities	persons with disabilities supported	No. of persons with disabilities receiving credit, loans	Loan & grant records	2026–2028	National Treasury, County Government
Entrepreneurship & incubation	Provide training & mentorship	persons with disabilities trained	No. of persons with disabilities completing training	Training reports	2026–2028	State Department for Trade, TVETA
	Establish incubation hubs	Hubs operational	No. of hubs established	Hub reports	2026–2028	State Department for Trade, Chambers of Commerce
Expand disability-sensitive social protection	Introduce cash transfers	persons with disabilities benefit	No. of persons with disabilities receiving transfers	Payment records	2026–2028	State Department for Social Protection, National Treasury

	Integrate pensions, insurance	Inclusive schemes expanded	% Of persons with disabilities enrolled	Insurance records	2026–2028	National Treasury, State Department for Social Protection
Simplify enrolment & outreach	Develop simplified systems	Enrolment streamlined	% reduction in barriers	Audit reports	2026–2026	State Department for Social Protection, County Government
	Conduct outreach campaigns	Awareness increased	No. of outreach sessions held	Campaign reports	2026–2028	OPDs, PBOs
Enforce anti-discrimination laws	Employer sensitization	Employers sensitized	No. of campaigns held	Campaign reports	2026–2028	State Department for Labour, FKE, KEEPSA
	Workplace monitoring	Inclusive hiring enforced	% of cases resolved	Inspection reports	2026–2028	State Department for Labour, HRC
Promote workplace accessibility	Provide assistive tech	Workplaces equipped	No. of workplaces supported	Procurement reports	2026–2028	State Department for Labour, MoH, FKE
	Adapt infrastructure	Infrastructure inclusive	No. of workplaces adapted	Inspection reports	2026–2028	State Department for Public Works, FKE
	Develop accommodation policies	Workplace policies adopted	No. of policies developed	Policy docs, HR manuals	2026–2028	State Department for Labour, FKE, KEEPSA

Key Result Area IV: Social Inclusion

Strategic Objective IV: To ensure the full inclusion of persons with disabilities in family and community life through access to personal assistance, opportunities for relationships, marriage and family, participation in culture, arts, recreation and sports, and equal access to justice, enabling them to enjoy their rights and contribute actively to society.

Strategy	Activity	Output	Output indicator	Means of verification	Time frame	Lead agency
A sustainable support system for caregivers through policy and programmes	Develop agreed standards for personal assistance	Standards for personal assistance	No. of Standards	Standards Developed	2026-2027	State Department for Social Protection (Directorate of Social Development)
	Develop and monitor appropriate personal assistance services	Appropriate personal assistance services	No. of personal assistance services No. of caregivers supported	Appropriate personal assistance services available	2026-2027	State Department for Social Protection (Directorate of Social Development)
	Promote respite care services	Respite care services	No. of respite care Service No. of caregivers supported	Respite care services available	2026-2027	State Department for Social Protection (Directorate of Social Development)
	Provide caregiver allowance	Feasibility study on caregiver allowance	No. of participants No. of Reports	Feasibility study on caregiver allowance undertaken	2026-2027	State Department for Social Protection (Directorate of Social Development)
Support persons with disabilities to have fulfilling relationships with members of their families and communities	Link persons with disabilities to appropriate support networks in the community.	Appropriate support networks in the community	No. of persons with disability linked No. of appropriate support networks	Persons with disability linked	2026-2027	State Department for Social Protection (Directorate of Social Development)
	Undertake disability sensitization training for health professionals on sexual and reproductive health services, including on family planning	Sensitization training for health professionals on sexual and reproductive health services, including on family planning.	No. of health professions sensitized No. of sensitization fora	Health professions sensitized	2026-2027	State Department for Social Protection (Directorate of Social Development)

	Link parents with children with disabilities to appropriate services and programmes to support parents with disabilities on their parenting roles.	Appropriate services and programmes that support parents with disabilities on their parenting roles.	No. of parents linked No. of appropriate services and programmes	Parents with children with disabilities linked	2026-2027	State Department for Social Protection (Directorate of Social Development)
Promote the use of culture and art for social change	Partner with artists and organize drama, arts and music events to challenge disability stigma and discrimination	Drama, arts and music events that challenge disability stigma and discrimination	No. of events	Events organized	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Support the positive portrayal of persons with disabilities and disability issues through cultural and artistic media	Media engagement to support the positive portrayal of persons with disabilities and disability issues.	No. of media shows No. of IEC materials	Media shows aired IEC material developed	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Develop and implement specific disability arts projects	Implement disability arts projects	No. of persons with disabilities supported No. of projects developed and implemented	Disability arts projects developed and implemented	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Promote dance, drama and music programmes as complementary types of therapy for persons with disabilities	Dance, drama and music programmes for persons with disabilities	No. of persons with disabilities enrolled No. of programmes	Persons with disabilities linked and enrolled.	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Promote disability inclusive women groups to increase the participation of women with disabilities in the community and mainstream disability.	Disability inclusive women groups	No. of women groups which are disability inclusive	Disability inclusive women groups registered	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Promote apprenticeship and employment opportunities for persons with disabilities within cultural and arts programmes	Apprentice and employment opportunities for persons with disabilities within cultural and arts programmes	No. of persons with disabilities onboarded No. of organizations or agencies	Persons with disabilities onboarded	2026 -2028	State Department for Social Protection (Directorate of Social Development)

Facilitate persons with disabilities to participate in local recreation, leisure, and sports.	Promote positive media coverage on disability inclusive recreation and sports	Positive media coverage on disability inclusive recreation and sports	No. of shows aired on disability inclusive recreation and sports	shows aired	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Promote inter-vocational sports days	Inter-vocational sports days	No. of participants	Inter-vocational sports days celebrated	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Link local disability recreation and sports groups, clubs to national and international organizations	Local disability recreation and sports groups, clubs linked to national and international organizations	No. of local disability recreation and sports groups, clubs supported	local disability recreation and sports groups, clubs supported	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Provide appropriate training and resources to support persons with disabilities who want to set up their own recreation and sports groups	Appropriate training and resources provided to support persons with disabilities who want to set up their own recreation and sports groups	No. of persons with disabilities trained and supported	Persons with disabilities trained and supported	2026 -2028	State Department for Social Protection (Directorate of Social Development)
Promote awareness of the rights of persons with disabilities	Sensitize persons with disabilities, caregivers, and intermediaries on access to justice systems	Persons with disabilities, caregivers and intermediaries sensitized on access to justice systems	No. of persons with disabilities, caregivers and intermediaries sensitized	Persons with disabilities, caregivers and intermediaries sensitized	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Sensitize Organizations of persons with disabilities (OPDs) Human Rights Organizations, and Community groups on disability rights	Organizations of persons with disabilities (OPDs), Human Rights Organizations, and Community groups sensitized on disability rights.	No. of Organizations of persons with disabilities (OPDs) Human Rights Organizations, and Self-Help Groups sensitized	Organizations of persons with disabilities (OPDs) Human Rights Organizations, and Community groups sensitized	2026 -2028	State Department for Social Protection (Directorate of Social Development)
	Train legal practitioners, administrators, law enforcement officials, medical personnel and other stakeholders on disability rights including case management involving persons with disabilities.	Legal practitioners, administrators, law enforcement officials, medical personnel and other stakeholders trained on disability rights including case management involving persons with disabilities	No. of legal practitioners, administrators, law enforcement officials, medical personnel and other stakeholders	legal practitioners, administrators, law enforcement officials, medical personnel and other stakeholders	2026 -2028	State Department for Social Protection (Directorate of Social Development)

Key Result Area V: Empowerment of persons with disabilities and their families

Strategic Objective V: To empower persons with disabilities through advocacy and communication, community mobilization, political participation, community groups and organizations of persons with disabilities as sustainable platforms for leadership, self-representation, and inclusive development.

Strategy	Activity	Output	Output indicator	Means of verification	Time frame	Lead agency
Sustained awareness and communication strategy that transforms public perceptions of persons with disabilities	Develop and disseminate disability awareness booklets, brochures and posters in public institutions.	Awareness publications distributed nationwide.	No. of copies distributed and institutions reached.	Distribution reports, institutional acknowledgment receipts.	2026-2028 (Continuous)	NCPWD, Ministry of Education, KNHCR, Media Council of Kenya
	Conduct nationwide radio and TV talk shows on disability rights and inclusion.	Media programmes aired on disability awareness.	No. of shows aired and audience reach ratings.	Broadcast schedules, media monitoring reports.	2026-2028 (Continuous)	State Department for Social Protection (Directorate of Social Development), Media Council of Kenya, NCPWD, Safaricom Foundation, Kenya Broadcasting Corporation, Private Sector,
	Integrate disability awareness modules into school curricula and community training programmes.	Educational modules developed and taught.	No. of schools and community centers adopting modules.	Curriculum documents, training reports.	2028	MoE, TSC, NCPWD, Kenya Institute of Curriculum Development (KICD), State Department for Social Protection (Directorate of Social Development), UN Agencies, Development Partners, Private Sector
	Organize public forums, exhibitions, and cultural events showcasing talents and innovations by persons with disabilities.	Public events conducted promoting positive disability image.	No. of events held and participants engaged.	Event reports, attendance registers, media coverage.	2026-2028 (Continuous)	State Department for Social Protection (Directorate of Social Development) NCPWD, County Governments, PBOs, Private Sector Foundations, UN Agencies, Development Partners
	Launch digital and social media campaigns highlighting success stories of persons with disabilities.	10 Digital campaigns implemented nationwide.	No. of campaigns launched online engagement statistics.	Social media analytics, campaign reports.	2026-2028 (Continuous)	OPDs, NCPWD, Safaricom, PBOs, UN Agencies, Development Partners, Private Sector, Bloggers Association of Kenya (BAKE)
Use of alternative communication methods to disseminate information to persons with disabilities and the community.	Develop and disseminate Braille, large print, and audio versions of key government and community information materials.	Accessible formats of information materials are available nationwide.	No. of Braille, large print, and audio materials produced and distributed.	Distribution reports, institutional acknowledgment receipts.	2026	NCPWD, State Department for Social Protection (Directorate of Social Development), Kenya Institute for the Blind, MoE, Kenya National Library Service, UN Agencies, Development Partners, Private Sector, OPDs
	Provide sign language interpretation services in public forums, media broadcasts, and government communication.	Sign language services are integrated into key communication channels.	No. of events and broadcasts with sign language interpretation.	Media monitoring reports, event records, feedback surveys.	2026	Kenya Sign Language Interpreters Association, NCPWD, Media Council of Kenya, Ministry of ICT, UN Agencies, Development Partners, Private Sector, OPDs

	Establish SMS and mobile app platforms with text-to-speech features for timely information sharing.	Digital platforms providing accessible communication services.	No. of users subscribed	Platform analytics, user feedback reports, system logs.	2026-2028	Safaricom, Communications Authority of Kenya, NCPWD, PBOs working on assistive technologies, UN Agencies, Development Partners, Private Sector, OPDs
Strengthened culture of participation among persons with disabilities in community groups	Conduct capacity-building workshops for persons with disabilities on group formation, leadership, and governance.	20 Workshops conducted and persons with disabilities trained on group participation and leadership.	No. of workshops held and participants trained.	Training reports, attendance lists, evaluation forms.	2026 - 2028	NCPWD, State Department for Social Protection (Directorate of Social Development), County Governments, OPDs, UN Agencies, Development Partners, Private Sector
	Develop and distribute training manuals and toolkits on community group participation tailored for persons with disabilities.	Accessible training manuals and toolkits produced and disseminated.	No. of manuals developed and distributed.	Manual copies, distribution records, feedback surveys.	2026 - 2028	State Department for Social Protection (Directorate of Social Development), KISE, PBOs, Kenya National Library Service, OPDs, UN Agencies, Development Partners, Private Sector
	Facilitate mentorship and peer-learning programmes linking experienced community leaders with persons with disabilities groups.	1 Mentorship and peer-learning programmes established. 20 Mentorship and peer-learning sessions conducted	No. of mentorship sessions conducted and programmes developed	Mentorship programme reports, beneficiary feedback, session records.	2026 - 2028	State Department for Social Protection (Directorate of Social Development) NCPWD, County Governments, PBOs, Community groups, UN Agencies, Development Partners, Private Sector, OPDs
Use existing community gathering structures and forums to engage and discuss community issues.	Leverage community barazas, chief's meetings, and ward forums to include discussions on disability and inclusion.	Community forums held with disability inclusion agenda integrated.	No. of forums held and proportion addressing disability issues.	Forum reports, attendance registers, meeting minutes.	2026-2028	State Department for Social Protection (Directorate of Social Development), County Governments, Ministry of Interior and National Administration, Community groups, OPDs
	Collaborate with faith-based and cultural institutions to include disability awareness in regular community gatherings and cultural events.	Faith-based and cultural events incorporating disability awareness sessions	Number of events and institutions engaged.	Event programmes, reports, media coverage, participant feedback.	2026 - 2028	State Department for Social Protection (Directorate of Social Development), NCPWD, KNCHR, NCCK, FBOs, CBOs, PBOs, UN Agencies, Development Partners, Private Sector, OPDs
Educate persons with disabilities on their rights to be involved through active participation in politics	Civic Education Workshops for persons with disabilities	10 civic education workshops conducted across various counties in Kenya targeting persons with disabilities.	No. of workshops conducted No. of persons with disabilities trained W	Workshop reports and attendance registers	2026 - 2028	State Department for Social Protection (Directorate of Social Development), IEBC, KNCHR, PBOs, HRs, UDPK, OPDs, NCPWD, UN Agencies, Development Partners, Private Sector
Promote representation of persons with disabilities in governance structures and process.	Facilitate the Nomination and Mentorship of persons with disabilities for Leadership Positions in County and National Government	Increased number of persons with disabilities nominated or appointed to decision-making and governance positions.	No. of mentorship sessions conducted No. of persons with disabilities mentored and supported in the nomination process	Mentorship program reports Testimonies or interviews	2026 - 2028	State Department for Social Protection (Directorate of Social Development), UDPK, OPDs, NGEC, NCPWD, PSC, PBOs, UN Agencies, Development Partners, Private Sector

Strengthen and resource organisations for persons with disabilities.	Conduct Capacity Building Workshops for OPDs on Governance, Leadership, and Financial Management	30 OPDs trained on governance, leadership, and financial management best practices.	No. of OPDs trained No. of workshops conducted	Training reports and attendance sheets Assessment results	2026 - 2028	State Department for Social Protection (Directorate of Social Development), UDPK, NCPWD, KNCHR, NGEC, PBOs, OPDs, UN Agencies, Development Partners, Private Sector
	Provide Grant Funding and Equipment Support to Strengthen OPD Operations	10 OPDs receive grant funding or office equipment to improve their operational capacity.	No. of OPDs receiving grants or equipment # of resources disbursed	Grant agreements Disbursement records Asset registers and delivery notes		State Department for Social Protection (Directorate of Social Development), NCPWD, OPDs, County Government, PBOs, UN Agencies, Development Partners, Private Sector

ANNEX II: MONITORING AND EVALUATION / RESULTS MATRIX FOR 2026-2030

Key Result Area I : Accessible health services.								
Outcomes:	Persons with disabilities achieve their highest attainable standard of health.							
Output	Output indicator	Unit of measure (Status, No.)	Baseline	Target for 3 years	Targets			Lead agency
					2026-27	2027-28	2028-29	
Comprehensive Health database, survey developed	1 Comprehensive Health database, survey	No.	0	1	1	0	0	Ministry of Health NCPWD
Community groups Registered and linked for persons with disabilities in the Community in CDMIS	50 groups Registered	No.	0	150	50	50	50	State Department for Social Protection (Directorate of Social Development) Ministry of Interior and National Government Administration
Trained Local artisans to make and repair assistive devices	150 Trained Local artisans	No.	0	450	150	150	150	State Department for Social Protection (Directorate of Social Development)
Improved facilities to access health care services to Persons with disabilities	50 Improved facilities	No.	0	150	50	50	50	Ministries, Departments, Agencies, private sector, partners, Organisation of and for Persons With Disabilities
Sensitization forums on reduction of risk of mothers and babies developing health conditions and, or impairments that may lead to disability.	3 sensitization forums	No.	0	9	3	3	3	Ministry of Health
Issue assistive devices to persons with disabilities	100 assistive devices issued	No.	0	300	100	100	100	State Department for Social Protection (Directorate of Social Development) NCPWD NFDK
Curriculum on disability streaming developed	1 Curriculum	No.	0	0	0	0	1	Ministry of Education Judiciary State Department for Social Protection (Directorate of Social Development)
Awareness forums created on issues on persons with disabilities	3 Awareness forums	No.	0	9	3	3	3	Ministries, Departments, Agencies, private sector, partners, organisation of and for persons with disabilities

Persons with disabilities attend a prevention programme	200 persons with disabilities empowered	No.	0	600	200	200	200	Ministry of Health
Prevention programmes developed	3 Prevention programmes developed			9	3	3	3	State Department for Social Protection (Directorate of Social Development), State Department for Labour
Resource materials, developed and distributed	500 Resource materials produced	No.	0	1500	500	500	500	Ministries, Departments, Agencies, private sector, partners, OPDs
Awareness and sensitization on persons with disability created	3 Awareness and sensitizations			9	3	3	3	
Health promotions and awareness conducted in the organisation	3 Health promotions and awareness conducted	No.	0	9	3	3	3	Ministries, Departments, Agencies, private sector, partners, OPDs
Trained health care providers on disability handling techniques	100 Trained health care providers	No.	0	300	100	100	100	Ministry of Health NCPWD
Trained health care providers on training on disability medical handle techniques	75 Trained health care providers	No.	0	225	75	75	75	Ministry of Health NCPWD
Trained CBID personnel to be Tots	140 trained CBID personnel	No.	0	420	140	140	140	State Department for Social Protection (Directorate of Social Development)
Individuals empowered with positive attitudes towards persons with disabilities	150 individuals empowered	No.	0	450	150	150	150	Ministries, Departments, Agencies, private sector, partners, OPDs
Persons with disabilities are empowered on their knowledge of healthcare services	150 persons with disabilities empowered	No.	0	450	150	150	150	Ministries, Departments, Agencies, private sector, partners, OPDs
Persons with disability are empowered in their healthy practices and lifestyle	150 persons with disabilities empowered	No.	0	450	150	150	150	Ministries, Departments, Agencies, private sector, partners, OPDs
Persons with disabilities are aware of the immunization programmes and immunized	150 persons with empowered on immunization and immunized programmes	No.	0	450	150	150	150	Ministry of Health Ministry of Interior and National Administration Ministry of Labour and Social Protection NCPWD
Sensitization forums on clean water and sanitation to persons with disability and the community done	3 sensitization forums on clean water and sanitation	No.	0	9	3	3	3	Ministry of Water, Sanitation and Irrigation Ministry of Labour and Social Protection
Screening of early identification of communicable or noncommunicable diseases for persons with disabilities done	3 Screening of early identification of communicable and noncommunicable diseases	No.	0	9	3	3	3	Ministry of Health Ministry of Interior and National Administration State Department for Social Protection (Directorate of Social Development), NCPWD

Awareness on self-management of chronic conditions to persons with disabilities created	2 awareness on self-management	No.	0	6	2	2	2	Ministry of Health Ministry of Interior and National Administration State Department for Social Protection (Directorate of Social Development) NCPWD
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Key Result Area II: Inclusive Education and Training

Outcomes:	Persons with disabilities have meaningful social roles and responsibilities in their families and communities and are treated as equal members of society.							
Output	Output indicator	Unit of measure (Status, No.)	Base-line	Target for 3 years	Targets			Lead agency
					2026-27	2027-28	2028-29	
Community accessibility Audit report	No. of transport operators trained on disability etiquette	No.	0	3 community accessibility audit reports	1	1	1	Ministry of Transport & Infrastructure, ICT, NCPWD, State Department for Social Protection (Directorate of Social Development), OPDs, County Governments
	No. of persons with disabilities reporting improved access.							
	No. of local artisans trained on accessible design							
	No. of community forums held on accessibility.							
	No. of public facilities upgraded for accessibility.							
Assistive technology distributed	No. of families receiving stipends	No.	0	450 families supported; 450 assistive devices distributed	150 families; 150 devices	150 families; 150 devices	150 families; 150 devices	NCPWD, State Department for Social Protection (Directorate of Social Development), MoE, MoH, County Governments
	No. of assistive devices subsidized or distributed							

Parents, caregiver financial literacy held	No. of parents and caregivers trained on financial literacy % increase in school attendance among supported learners	No.	0	300 parents, caregivers trained	100	100	100	NCPWD, State Department for Social Protection (Directorate of Social Development), MoE, MoH, County Governments
Adapted curriculum modules developed	No. of adapted curriculum modules developed	No.	0	3 adapted curriculum modules	1	1	1	MoE, KICD, KISE, NITA, State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments
Speech therapy sessions conducted	No. of speech therapy sessions conducted No. of occupational therapy sessions delivered No. of counselling sessions provided	No.	0	900 sessions (speech, OT, counselling)	300	300	300	MoE, MoH, State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments, Physiotherapy Council of Kenya
Occupational therapy sessions delivered	Number of occupational therapy sessions delivered	No.	0	900 occupational therapy sessions	300	300	300	MoE, MoH, State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments, Kenya Occupational Therapists Associations
Counselling sessions provided	No. of counselling sessions provided	No.	0	900 counselling sessions	300	300	300	MoE, MoH, State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments
Peer support groups established	Number of peer support groups established	No.	0	30 peer support groups established	0	15	15	NCPWD, State Department for Social Protection (Directorate of Social Development), County Governments, CBOs, OPDs
Learners demonstrating progress in communication, motor skills, or emotional well-being	Percentage of learners showing improvement in targeted areas	%	0%	60% learners showing improvement	20%	40%	60%	MoE, MoH, State Department for Social Protection (Directorate of Social Development), NCPWD, County Governments

A standardized training curriculum and delivery framework developed and implemented.	No. of educators trained using the standardized curriculum. No. of counties covered by the training program.	No.	0	1 curriculum; 600 educators trained; 47 counties covered	1 curriculum; 200 educators; 15 counties	200 educators; 16 counties	200 educators; 16 counties	MoE, KICD, NITA, State Department for Social Protection (Directorate of Social Development), NCPWD, OPDs, KISE, County Governments
10 Functional resource centers equipped with assistive technologies and inclusive education materials established	No. of resource hubs established and operational.	No.	0	10 resource hubs established and operational	3	4	3	MoE, KISE, KICD, NITA, State Department for Social Protection (Directorate of Social Development), NCPWD, OPDs, County Governments
3 Early identification and referral systems established.	Number of assessment centers established and operational. Number of children screened for developmental delays or disabilities.	No.	0	3 assessment centers; 3,000 children screened	1 center; 1,000 children screened	1 center; 1,000 children	1 center; 1,000 children screened	MoH, MoE, County Governments, NCPWD, State Department for Social Protection (Directorate of Social Development), Kenya Pediatrics Association, OPDs
Trained frontline workers (CHVs and ECDE teachers) equipped with skills	· Number of CHVs and ECDE teachers trained. · Number of children identified early and receiving support services.	No.	0	600 CHVs and ECDE teachers trained; 1,500 children identified early	200 trained; 500 children identified	200 trained; 500 children identified	200 trained; 500 children identified	MoH, MoE, County Governments, NCPWD, State Department for Social Protection (Directorate of Social Development), OPDs
Disability-responsive assessment tools developed	No. of inclusive assessment tools developed	No.	0	3 inclusive assessment tools developed	1	1	1	MoH, MoE, County Governments, NCPWD, State Department for Social Protection (Directorate of Social Development), OPDs
Disability-responsive assessment tools piloted	No. of schools and learners involved in the pilot.	No.	0	15 schools; 300 learners in pilot	0	5 schools; 100 learners	10 schools; 200 learners	MoH, MoE, KISE, KNEC, KICD, County Governments, NCPWD, State Department for Social Protection (Directorate of Social Development), OPDs
Teachers and examiners trained in adapted assessment techniques	No. of educators and assessors trained. No. of schools implementing inclusive assessments.	No.	0	300 educators and assessors trained; 15 schools with inclusive assessments	0	150 trained; 5 schools	150 trained; 10 schools	MoH, MoE, KISE, KNEC, KICD, County Governments, NCPWD, State Department for Social Protection (Directorate of Social Development), OPDs, TSC

Key Result Area III: Livelihood, Social Protection, and Community Development Persons with disabilities achieve their highest attainable standard of health

Outcomes:	Persons with disabilities have meaningful social roles and responsibilities in their families and communities and are treated as equal members of society.							
Output	Output indicator	Unit of measure (Status, No.)	Baseline	Target for 3 years	Targets			Lead agency
					2026-27	2027-28	2028-29	
Monographs on 7 disability domains developed	No. of monographs produced & disseminated	No.	0	7 monographs	3	4	Consolidation	MoH, State Department for Social Protection (Directorate of Social Development)
National Assistive Technology Policy developed	Policy submitted to parliament for approval	Status (Yes, No)	Draft policy exists	1 policy approved	Policy finalized & submitted	Approval process	Implementation roadmap	MIT, NCPWD, Social Protection
Reasonable accommodation provided in workplaces	No. of workplaces adopting disability-friendly adjustments	No.	<10 workplaces	100 workplaces	20	40	40	MoL, Employers Federation
National disability policy approved	Policy submitted to parliament for approval	Status (Yes, No)	Draft exists	1 national policy approved	Submission	Approval	Implementation rollout	State Department for Social Protection (Directorate of Social Development), Parliament
Assistive technology provided to persons with disabilities	No. of persons receiving assistive technology	No.	<10% persons with disabilities covered	50,000 persons with disabilities	15,000	20,000	15,000	MoH, NCPWD
Assistive devices issued to persons with disabilities	No. of persons issued with assistive devices	No.	<15% persons with disabilities covered	60,000 persons with disabilities	20,000	20,000	20,000	MoH, PBOs, OPDs
Community Groups empowered with start-up capital	No. of Community Groups supported	No.	Few Community Groups functional	200 Community Groups supported	50	75	75	Gender, County Government
OPDs trained on rights	No. of OPDs trained	No.	20 OPDs trained	100 OPDs trained	30	35	35	NCPWD, OPDs
Community Groups trained on business development	No. of groups trained	No.	Few trained	150 Community Groups trained	50	50	50	Ministry of Trade, PBOs
Disability cash transfer piloted	No. of pilot counties	Status (Yes, No)	None	2 counties piloted	1 county	1 county	Evaluation	National Treasury, Social Protection
Disability-friendly financial products developed	No. of products developed	No.	None	5 products developed	2	2	1	National Treasury, Micro-National Treasury Institutions
Persons with disabilities entrepreneurs supported financially	No. Persons with disabilities of accessing grants, loans	No.	<5% Persons with disabilities access	20,000 s Persons with disabilities supported	5,000	7,500	7,500	National Treasury, County Government

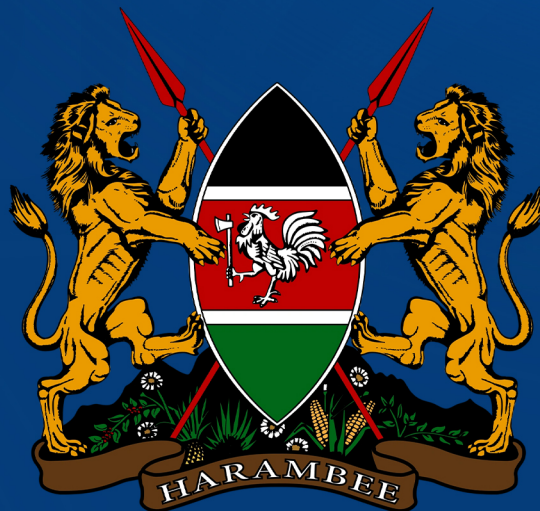
Persons with disabilities trained on entrepreneurship	No. of Persons with disabilities completing training	No.	<1,000 trained	10,000 Persons with disabilities trained	3,000	3,500	3,500	Trade, TVET
Business incubation hubs established	No. of hubs operational	No.	None	10 hubs	3	3	4	Trade, Chambers of Commerce
Persons with disabilities benefiting from disability cash transfers	No. of Persons with disabilities receiving transfers	No.	<2% coverage	100,000 Persons with disabilities	30,000	35,000	35,000	Social Protection, National Treasury
Persons with disabilities enrolled in pensions & insurance	% Of Persons with disabilities enrolled	%	<5% enrolled	25% Persons with disabilities enrolled	10%	18%	25%	National Treasury, Social Protection
Simplified enrolment systems established	% reduction in barriers	%	Complex, exclusionary	50% reduction in barriers	20%	35%	50%	Social Protection, County Government
Outreach campaigns conducted	No. of outreach sessions held	No.	Few conducted	300 sessions	100	100	100	OPDs, PBOs
Employers sensitized on disability rights	No. of campaigns conducted	No.	Limited	200 campaigns	70	65	65	Labour, Employers Federation
Inclusive hiring practices enforced	% of reported cases addressed	%	Low enforcement	70% cases resolved	30%	50%	70%	Labour, HRC
Workplaces equipped with assistive technology	No. of workplaces supported	No.	Few workplaces accessible	200 workplaces	70	70	60	Labour, MoH, Employers Federation
Persons with disabilities enrolled in pensions & insurance	% Of Persons with disabilities enrolled	%	<5% enrolled	25% Persons with disabilities enrolled	10%	18%	25%	National Treasury, Social Protection

Key Result Area IV: Social Inclusion.

Outcomes:	Persons with disabilities have meaningful social roles and responsibilities in their families and communities and are treated as equal members of society.							
Output	Output indicator	Unit of measure (Status, No.)	Baseline	Target for 3 years	Targets			Lead agency
					2026-27	2027-28	2028-29	
Standards developed	8 stakeholder engagements	No.	0	8		4	4	ML & SP
Respite care centres established	8 respite care centres	No.	0	8		4	4	ML & SP
Cost analysis on caregiver allowance undertaken	1 Cost analysis report	No.	0	1		1	0	ML & SP
Peer support and counselling strengthened	100 of peer support groups formed.	No.	0	100	0	50	50	ML & SP
Persons with disabilities linked to support networks	150 persons with disabilities linked	No.	0	150	0	75	75	ML & SP
CBID, CBR Volunteers enlisted and empowered.	1170 CBID, CBR volunteers	No.	0	1170	390	390	390	ML & SP
Parents empowered	1200 parents trained	No.	0	1200	400	400	400	ML & SP
Drama, arts and music events organized	50 drama, arts and music events organized	No.	0	50	10	20	20	ML & SP
Inclusive cultural, sporting and recreation programmes developed	3 of programmes developed	No.	0	3	0	2	1	ML & SP
Culture and arts programme developed	800 students with disabilities accessing apprenticeship	No.	0	800	0	400	400	ML & SP
Positive media coverage facilitated	12 positive media coverage activities	No.	0	12	0	6	6	ML & SP
Inter VRC sports days organized	3 sports days	No.	0	3	1	1	1	ML & SP
Disability recreation and sports groups linked	60 local disability groups linked	No.	0	60	0	30	30	ML & SP
Persons with disabilities, caregivers, and intermediaries sensitized.	900 persons with disabilities, caregivers, and intermediaries	No.	0	900	0	300	300	Judiciary
Awareness raising undertaken	300 awareness raising forums	No.	0	300	100	100	100	ML & SP
Disability awareness carried out.	500 Health professionals trained	No.	0	500	0	250	250	MoH

Key Result Area V: Empowerment of persons with disabilities and their families.

Outcomes:	Persons with disabilities have meaningful social roles and responsibilities in their families and communities and are treated as equal members of society.							
Output	Output indicator	Unit of measure (Status, No.)	Baseline	Target for 3 years	Targets			Lead agency
					2026-27	2027-28	2028-29	
Monographs on 7 disability domains developed	No. of monographs produced & disseminated	No.	0	7 monographs	3	4	Consolidation	MoH, State Department for Social Protection (Directorate of Social Development)
National Assistive Technology Policy developed	Policy submitted to parliament for approval	Status (Yes, No)	Draft policy exists	1 policy approved	Policy finalized & submitted	Approval process	Implementation roadmap	MIT, NCPWD, Social Protection
Reasonable accommodation provided in workplaces	No. of workplaces adopting disability-friendly adjustments	No.	<10 workplaces	100 workplaces	20	40	40	MoL, Employers Federation
National disability policy approved	Policy submitted to parliament for approval	Status (Yes, No)	Draft exists	1 national policy approved	Submission	Approval	Implementation rollout	State Department for Social Protection (Directorate of Social Development), Parliament
Assistive technology provided to Persons with disabilities	No. of persons receiving assistive technology	No.	<10% persons with disabilities covered	50,000 persons with disabilities	15,000	20,000	15,000	MoH, NCPWD
Assistive devices issued to Persons with disabilities	No. of persons issued with assistive devices	No.	<15% persons with disabilities covered	60,000 persons with disabilities	20,000	20,000	20,000	MoH, PBOs, OPDs
Community groups empowered with start-up capital	No. of Community groups supported	No.	Few Community Groups functional	200 Community Groups supported	50	75	75	State Department for Social Protection (Directorate of Social Development), County Government



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